

#1 · Food and Nutrition Expertise

What are the two active transport mechanisms for monosaccharide absorption in the small intestine?

Answer

Glucose and galactose are absorbed via sodium-dependent active transport (SGLT1), while fructose is absorbed via facilitated diffusion (GLUT5).

#2 · Food and Nutrition Expertise

What is the physiological role of soluble dietary fibre after it reaches the colon?

Answer

Soluble fibre is fermented by gut bacteria into short-chain fatty acids, which support colonocyte health and may favourably influence lipid metabolism and glycemic response.

#3 · Food and Nutrition Expertise

What two protein quality scoring systems does a Canadian dietitian use to evaluate plant-based diets?

Answer

The Protein Digestibility Corrected Amino Acid Score (PDCAAS) and the Digestible Indispensable Amino Acid Score (DIAAS) both measure how well a protein source supplies indispensable amino acids in digestible form.

#4 · Food and Nutrition Expertise

What structure transports absorbed dietary fat from the enterocyte into circulation?

Answer

Reassembled triglycerides are packaged into chylomicrons within the enterocyte and transported first through the lymphatic system before entering the bloodstream.

#5 · Food and Nutrition Expertise

Which two fatty acids are considered essential and must come from the diet?

Answer

Linoleic acid (an omega-6 fatty acid) and alpha-linolenic acid (an omega-3 fatty acid) cannot be synthesized by the body and must be obtained from food sources such as vegetable oils, nuts, seeds, and fatty fish.

#6 · Food and Nutrition Expertise

Why are fat-soluble vitamins a greater toxicity concern than water-soluble vitamins?

Answer

Fat-soluble vitamins (A, D, E, K) are stored in the liver and adipose tissue rather than being readily excreted, so excessive supplementation can accumulate to toxic levels over time.

#7 · Food and Nutrition Expertise

Why is vitamin D status a particular concern in Canadian dietetic practice?

Answer

Limited sun exposure through much of the year at northern latitudes reduces cutaneous vitamin D synthesis, so Health Canada recommends routine consideration of intake or supplementation, especially for older adults, breastfed infants, and people with limited sun exposure.

#8 · Food and Nutrition Expertise

Why does Health Canada mandate folic acid fortification of white flour, pasta, and cornmeal?

Answer

Adequate folate intake before and during early pregnancy reduces the risk of neural tube defects, so mandatory fortification helps protect the general population, including pregnancies that may not yet be recognized.

#9 · Food and Nutrition Expertise

Who is at elevated risk of vitamin B12 deficiency and why?

Answer

Individuals who have had gastric surgery, strict vegans, and older adults with reduced gastric acid production are at higher risk, because B12 absorption depends on intrinsic factor from gastric parietal cells and on adequate stomach acid to release B12 from food protein.

#10 · Food and Nutrition Expertise

What is the key difference between heme and non-heme dietary iron absorption?

Answer

Heme iron, from animal tissue, is absorbed efficiently via a dedicated transporter, while non-heme iron, from plant sources, is absorbed less efficiently and is influenced by enhancers like vitamin C and inhibitors like phytates and tannins.

#11 · Food and Nutrition Expertise

Why might reducing sodium intake inadvertently reduce iodine intake in Canada?

Answer

Iodine is supplied in Canada primarily through iodized table salt, so clients reducing sodium for blood pressure management may also reduce their iodine intake unless counselled to choose iodized salt in moderation or other iodine sources.

#12 · Food and Nutrition Expertise

How does long-term proton pump inhibitor use affect nutrient status?

Answer

Proton pump inhibitors reduce gastric acid production, which can impair the absorption of vitamin B12, calcium, and iron with long-term use.

#13 · Food and Nutrition Expertise

What dietary counselling approach is used for a client on warfarin therapy?

Answer

Because vitamin K antagonizes warfarin's anticoagulant effect, dietitians counsel for consistency in day-to-day vitamin K intake rather than eliminating vitamin K-rich foods entirely.

#14 · Food and Nutrition Expertise

How does grapefruit juice interact with certain medications?

Answer

Grapefruit juice inhibits the cytochrome P450 3A4 enzyme in the intestinal wall, which can raise blood levels of many statins and calcium channel blockers.

#15 · Food and Nutrition Expertise

What food-medication interaction is a concern for clients taking monoamine oxidase inhibitors?

Answer

Tyramine-containing aged, fermented, or cured foods can precipitate a hypertensive crisis in clients taking monoamine oxidase inhibitors, so intake of these foods must be limited.

#16 · Food and Nutrition Expertise

What is the Estimated Average Requirement (EAR) used for in dietetic practice?

Answer

The EAR is the intake level estimated to meet the needs of half the healthy individuals in a life stage and sex group, and it is the value used to assess the adequacy of a group's intake.

#17 · Food and Nutrition Expertise

What is the Recommended Dietary Allowance (RDA) and when is it used?

Answer

The RDA is set at roughly the EAR plus two standard deviations to meet the needs of nearly all healthy individuals in a group, and it is used to plan and evaluate intake for individual clients.

#18 · Food and Nutrition Expertise

When is an Adequate Intake (AI) value used instead of an RDA?

Answer

An AI is used when there is insufficient evidence to set an Estimated Average Requirement, representing an observed or approximated intake level assumed to be adequate.

#19 · Food and Nutrition Expertise

What does the Tolerable Upper Intake Level (UL) represent?

Answer

The UL is the highest average daily nutrient intake likely to pose no risk of adverse health effects to almost all individuals, and it is particularly relevant when assessing supplement or fortified food use.

#20 · Food and Nutrition Expertise

What are the Acceptable Macronutrient Distribution Ranges (AMDR) for adults?

Answer

For adults, the AMDR is roughly 45 to 65 percent of energy from carbohydrate, 10 to 35 percent from protein, and 20 to 35 percent from fat.

#21 · Food and Nutrition Expertise

How does Canada's Food Guide (2019) divide the plate?

Answer

Half the plate is vegetables and fruits, one quarter is protein foods with an emphasis on plant-based sources, and one quarter is whole grain foods, with water recommended as the drink of choice.

#22 · Food and Nutrition Expertise

How did the 2019 Canada's Food Guide change from the previous edition's approach?

Answer

It moved away from four food groups and prescribed serving sizes toward a plate-based visual and broader healthy eating behaviour guidance, such as cooking more often and eating mindfully.

#23 · Food and Nutrition Expertise

What behaviours, beyond food choice, does the 2019 Canada's Food Guide emphasize?

Answer

It emphasizes cooking more often, eating meals mindfully, enjoying food, eating with others when possible, using food labels, and being mindful of food marketing influence.

#24 · Food and Nutrition Expertise

What is the Canadian Nutrient File?

Answer

The Canadian Nutrient File, maintained by Health Canada, is the primary reference database of nutrient values for foods commonly consumed in Canada and underlies most Canadian nutrient analysis software.

#25 · Food and Nutrition Expertise

What percent Daily Value thresholds indicate a food is low or high in a nutrient?

Answer

A percent Daily Value of 5 percent or less is considered low in that nutrient, while 15 percent or more is considered high, as shown on the Nutrition Facts table.

#26 · Food and Nutrition Expertise

Why must nutrient retention during cooking be considered in food composition analysis?

Answer

Heat-sensitive water-soluble vitamins such as vitamin C and folate can be significantly reduced during cooking, so analyzing a prepared recipe requires accounting for retention factors rather than using raw ingredient values alone.

#27 · Food and Nutrition Expertise

Why is a randomized controlled trial generally considered stronger evidence than an observational cohort study?

Answer

Randomization helps control for confounding variables, allowing researchers to more confidently attribute an outcome to the intervention rather than to an unmeasured factor, whereas observational studies can only show association.

#28 · Food and Nutrition Expertise

What is recall bias and why does it matter in dietary research?

Answer

Recall bias occurs when participants inaccurately remember or report their past dietary intake, which can distort the results of studies relying on self-reported food records or dietary recalls.

#29 · Food and Nutrition Expertise

What is the Mifflin-St Jeor equation used for?

Answer

It is a predictive equation commonly used in Canadian clinical practice to estimate resting energy expenditure from weight, height, age, and sex, then adjusted with an activity factor to estimate total energy needs.

#30 · Food and Nutrition Expertise

Why is indirect calorimetry preferred over predictive equations in critical illness?

Answer

Predictive equations are known to be less reliable in complex clinical situations such as critical illness, so a measured value from indirect calorimetry provides a more accurate estimate of energy expenditure.

#31 · Food and Nutrition Expertise

Why is excess sodium intake a Canadian public health nutrition priority?

Answer

Sodium is the primary determinant of extracellular fluid volume, and excess intake, common with processed and restaurant foods, is linked to elevated blood pressure in sodium-sensitive individuals, prompting Health Canada's sodium reduction strategy.

#32 · Food and Nutrition Expertise

What is the clinical significance of potassium as the principal intracellular cation?

Answer

Potassium supports normal cardiac and neuromuscular function, and both hypokalemia and hyperkalemia carry cardiac risk, making potassium status important in renal and cardiac dietetic care.

#33 · Food and Nutrition Expertise

What condition results from acute thiamin deficiency affecting the nervous system?

Answer

Wernicke's encephalopathy is the acute neurological form of thiamin deficiency and is a medical emergency requiring prompt recognition and treatment.

#34 · Food and Nutrition Expertise

How does soaking or fermenting legumes and grains improve mineral absorption?

Answer

These preparation methods reduce phytate content, and since phytates inhibit mineral absorption (particularly zinc and iron), reducing them improves the bioavailability of these minerals.

#35 · Food and Nutrition Expertise

What three laboratory markers does a dietitian interpret together to assess iron status?

Answer

Serum ferritin, transferrin saturation, and hemoglobin are interpreted together with dietary history to distinguish depleted iron stores, iron-deficient erythropoiesis, and overt iron-deficiency anemia.

#36 · Professionalism and Ethics

What is a provincial dietetic regulatory college's core function?

Answer

To protect the public interest by setting standards of practice, registering qualified applicants, investigating complaints, and disciplining members whose conduct falls below the expected standard.

#37 · Professionalism and Ethics

How does a provincial regulatory college differ from Dietitians of Canada?

Answer

The college has legal authority under provincial health professions legislation to license and discipline registrants, while Dietitians of Canada is the national professional association that advocates for the profession and supports members without disciplinary authority.

#38 · Professionalism and Ethics

What is self-regulation in the context of dietetics?

Answer

The privilege, granted by government, for the dietetic profession to set its own standards and discipline its own members, in exchange for a reciprocal obligation to place public protection ahead of the profession's own interests.

#39 · Professionalism and Ethics

Which province does not use the CDRE for dietitian registration?

Answer

Quebec, which has its own separate registration pathway through the Ordre professionnel des dietetistes du Quebec.

#40 · Professionalism and Ethics

What is the difference between standards of practice and the Code of Ethics?

Answer

Standards of practice are specific, measurable, and enforceable expected behaviours, while the Code of Ethics provides the underlying values and principles that guide judgment in situations the standards do not explicitly address.

#41 · Professionalism and Ethics

How is scope of practice for dietitians defined in Canada?

Answer

Scope of practice is defined provincially and can vary slightly between jurisdictions, although the core competencies for RD registration are established nationally and are consistent across Canada.

#42 · Professionalism and Ethics

Name the four core values in the Canadian dietetic Code of Ethics.

Answer

Client wellbeing, client self-determination, integrity in professional relationships, and responsible use of knowledge and skill.

#43 · Professionalism and Ethics

What does client self-determination require of an RD?

Answer

Respecting the client's right to make their own informed decisions about their care, even if the RD disagrees, provided the client has capacity and has received the information needed to decide meaningfully.

#44 · Professionalism and Ethics

What should an RD do when a capable client insists on an approach the RD believes is unsafe?

Answer

Provide full, evidence-based information, document the discussion, respect the client's final decision, avoid coercion, and continue to offer support, since the RD cannot override a capable adult's autonomous decision.

#45 · Professionalism and Ethics

What does responsible use of knowledge and skill require of an RD?

Answer

Practising only within demonstrated competence, staying current with evolving evidence, and representing one's qualifications accurately.

#46 · Professionalism and Ethics

List the four elements required for valid informed consent.

Answer

The consent must be specific to the proposed treatment, given voluntarily without coercion, informed with understanding of risks and alternatives, and given by a person with the capacity to consent.

#47 · Professionalism and Ethics

What does it mean that capacity is decision-specific and time-specific?

Answer

A client may have capacity to consent to a simple intervention while lacking capacity for a more complex one, and capacity can change over time due to illness or cognitive status.

#48 · Professionalism and Ethics

What should an RD do when a client lacks capacity to consent?

Answer

Identify and work with the appropriate substitute decision-maker as defined by provincial legislation, following the hierarchy set out in that legislation.

#49 · Professionalism and Ethics

What is the difference between express and implied consent?

Answer

Express consent is explicitly stated verbally or in writing, while implied consent is inferred from the circumstances, such as a client voluntarily providing assessment information.

#50 · Professionalism and Ethics

Can a client withdraw consent partway through an intervention?

Answer

Yes, consent is an ongoing process, and the RD must respect a client's withdrawal of consent at any time, even mid-intervention.

#51 · Professionalism and Ethics

What legislation in Ontario governs health information custodians' handling of personal health information?

Answer

The Personal Health Information Protection Act (PHIPA), which governs how health information custodians, including RDs providing health care, collect, use, retain, and disclose personal health information.

#52 · Professionalism and Ethics

What is PIPEDA and when does it apply to dietitians?

Answer

The Personal Information Protection and Electronic Documents Act governs collection, use, and disclosure of personal information by private-sector organizations engaged in commercial activity, applying to many private-practice dietitians where no substantially similar provincial law takes precedence.

#53 · Professionalism and Ethics

What is the 'circle of care'?

Answer

The group of health professionals directly involved in a client's treatment who may share health information among themselves without requiring separate express consent for each disclosure, because such sharing is implied by the client's decision to seek care.

#54 · Professionalism and Ethics

When can personal health information be disclosed without client consent?

Answer

In specific legally defined circumstances, such as a duty to report certain communicable diseases to public health, a court order, or a genuine and imminent risk of serious harm to the client or an identifiable third party.

#55 · Professionalism and Ethics

What should an RD do after discovering a privacy breach, such as leaving a chart visible to other patients?

Answer

Report the breach according to the employer's and regulatory college's breach protocols, which in many provinces also requires notifying the client and, for serious breaches, the relevant privacy commissioner.

#56 · Professionalism and Ethics

Is discussing a client with a family member who is not a substitute decision-maker permitted without consent?

Answer

No, sharing information outside the circle of care, including with family members who are not substitute decision-makers, requires explicit client consent unless a specific legal exception applies.

#57 · Professionalism and Ethics

Define conflict of interest in dietetic practice.

Answer

A situation where a dietitian's personal, financial, or other interests could improperly influence, or appear to influence, their professional judgment or the objectivity of advice given to a client.

#58 · Professionalism and Ethics

Give an example of a common conflict of interest in dietetic practice.

Answer

Selling supplements directly to clients for personal profit, or receiving payment or gifts from a company whose products the RD then recommends.

#59 · Professionalism and Ethics

What is the operative principle for managing a disclosed conflict of interest?

Answer

Transparency, so that a client who is fully informed of a financial relationship can decide, with that knowledge, whether to proceed with the RD's services.

#60 · Professionalism and Ethics

Is failing to disclose a conflict of interest still a breach if the advice given is sound?

Answer

Yes, because non-disclosure deprives the client of information relevant to evaluating the advice, independent of the outcome.

#61 · Professionalism and Ethics

What core activities fall within most Canadian RDs' scope of practice?

Answer

Nutrition assessment, diagnosis of nutrition problems, and nutrition intervention.

#62 · Professionalism and Ethics

Give an example of an expanded scope activity some provinces authorize for RDs.

Answer

Ordering specific laboratory tests relevant to nutrition care, or adjusting insulin doses within a collaborative care plan, typically requiring additional certification beyond baseline RD registration.

#63 · Professionalism and Ethics

Why must an RD confirm exactly what expanded scope activities their own province authorizes?

Answer

Because expanded scope activities vary by province and require additional certification, so an activity permitted in one province may not be authorized in another.

#64 · Professionalism and Ethics

How should an RD approach a texture-modified diet decision that also involves a speech-language pathologist?

Answer

Through collaborative communication respecting both professionals' overlapping expertise, rather than one professional unilaterally overriding the other's recommendations.

#65 · Professionalism and Ethics

What happens when a dietitian practises outside their authorized scope?

Answer

It exposes both the client and the dietitian to risk and may result in disciplinary consequences from the regulatory college, regardless of the actual outcome.

#66 · Professionalism and Ethics

What does professional accountability mean for an RD?

Answer

Each RD is personally responsible for their own practice decisions and cannot delegate that responsibility to a supervisor, employer, or institutional policy.

#67 · Professionalism and Ethics

Is 'my employer told me to' a valid defence against a finding of professional misconduct?

Answer

No, regulatory colleges consistently hold that following an employer's conflicting policy is not a valid defence if it results in a breach of a professional standard.

#68 · Professionalism and Ethics

What should an RD do if an employer's policy conflicts with a professional standard of practice?

Answer

Raise the concern, document it, and, if the conflict cannot be resolved, decline to act in a manner that would breach the standard, even at professional or employment risk.

#69 · Professionalism and Ethics

What is a continuing competence program?

Answer

A required cyclical process of self-assessment against defined competencies, development of a learning plan, completion of continuing education, and periodic college audit, required by every Canadian dietetic regulatory college.

#70 · Professionalism and Ethics

Can registration be suspended solely for failing continuing competence requirements?

Answer

Yes, failure to complete continuing competence requirements can result in suspension of registration even without any specific complaint about clinical practice.

#71 · Professionalism and Ethics

What is the professionally correct response after an RD makes a clinical error, such as a feeding order calculation mistake?

Answer

Disclose the error to the appropriate parties, document what happened, and take steps to mitigate harm, rather than concealing the error.

#72 · Professionalism and Ethics

How do regulatory colleges generally treat honest disclosure of an error compared to concealment?

Answer

Honest disclosure and remediation are treated far more favourably in a disciplinary context, while concealment independently constitutes a serious integrity breach.

#73 · Professionalism and Ethics

Why do professional boundaries exist in the dietitian-client relationship?

Answer

To protect clients from harm arising from the inherent power imbalance created by the dietitian's specialized knowledge and potential influence over care decisions.

#74 · Professionalism and Ethics

Is a sexual or romantic relationship with a current client permitted under Canadian dietetic regulation?

Answer

No, it is universally prohibited by Canadian dietetic regulatory colleges and typically also prohibited for a defined period after the professional relationship ends.

#75 · Professionalism and Ethics

Give an example of a subtle boundary crossing in dietetic practice.

Answer

Accepting significant personal gifts from a client, developing a dual relationship such as becoming a client's business partner, or excessive self-disclosure that shifts focus away from the client.

#76 · Professionalism and Ethics

Why might accepting a current client's personal social media friend request be a boundary or privacy concern?

Answer

It can blur the professional relationship and risks unintended disclosure of personal or health information depending on the platform and content involved.

#77 · Professionalism and Ethics

What must an RD do to ethically terminate a professional relationship with a client?

Answer

Give reasonable notice, avoid abandonment particularly for clients with ongoing or urgent nutrition needs, and support continuity of care such as providing a summary to the receiving provider with consent.

#78 · Professionalism and Ethics

What underlying bioethical principles does client wellbeing in the Code of Ethics reflect?

Answer

Beneficence, acting to benefit the client, and non-maleficence, avoiding causing harm.

#79 · Professionalism and Ethics

What is the typical exam-tested response when a client discloses risk of serious harm to themselves or another identifiable person?

Answer

Follow the documented legal and regulatory duty-to-warn or duty-to-report process rather than ignoring the disclosure or acting solely on personal judgment outside that process.

#80 · Professionalism and Ethics

What is the appropriate first step if an RD has concerns about a colleague's practice falling below standard?

Answer

Raise the concern through the appropriate internal or regulatory college complaint pathway rather than ignoring it or confronting the colleague informally without following process.

#81 · Professionalism and Ethics

How should an RD respond to a request from a client's family member for health information without the client's consent?

Answer

Decline to share the information unless the family member is a substitute decision-maker or another legal exception applies, and explain the confidentiality obligation.

#82 · Professionalism and Ethics

What is the recommended response when organizational pressure conflicts with an RD's clinical judgment, such as pressure for early discharge?

Answer

Advocate for the client's documented clinical needs, raise the concern through appropriate channels, and avoid discharging or altering care solely to satisfy organizational pressure.

#83 · Professionalism and Ethics

What kind of exam answer options are almost always incorrect in professionalism and ethics scenarios?

Answer

Options involving ignoring a concern, acting on assumption without confirming facts, bypassing consent requirements for convenience, or resolving a conflict of interest through non-disclosure.

#84 · Professionalism and Ethics

What is the relationship between provincial health professions legislation and a regulatory college's authority?

Answer

Provincial health professions legislation, such as the Regulated Health Professions Act in Ontario, grants the college its legal authority to register, set standards for, and discipline dietitians.

#85 · Professionalism and Ethics

Why is documentation of a consent discussion important?

Answer

It protects both the client and the practitioner by providing evidence of what information was provided and how the client responded, and is itself a standard of practice requirement.

#86 · Professionalism and Ethics

What level of consent is generally appropriate for routine nutrition counselling versus a higher-risk intervention?

Answer

Routine counselling may rely on implied consent established at the outset of the relationship, while higher-risk or more invasive interventions generally warrant express, often written, consent.

#87 · Professionalism and Ethics

What must RDs ensure regarding staff or students they supervise and confidentiality?

Answer

That any staff or students under their supervision understand and follow the same confidentiality obligations as the RD.

#88 · Professionalism and Ethics

What steps should an RD take to secure client records?

Answer

Take reasonable steps to secure paper and electronic records against unauthorized access and use secure methods when transmitting sensitive health information, avoiding unencrypted email for sensitive details.

#89 · Professionalism and Ethics

Why can refusing to perform an activity within one's authorized and demonstrated competence also be a professionalism concern?

Answer

Because excessive caution that results in unnecessary delay or harm to the client can itself represent a failure to meet professional obligations to the client.

#90 · Professionalism and Ethics

What is the core reciprocal obligation that comes with self-regulation?

Answer

The profession must consistently demonstrate that it acts in the public interest ahead of its own interests, in exchange for the privilege of setting its own standards.

#91 · Professionalism and Ethics

How does the CDRE typically frame ethics and professionalism items?

Answer

As realistic scenarios requiring the candidate to identify the most appropriate action among several plausible options, rather than recall of an isolated definition.

#92 · Professionalism and Ethics

What does providing services to a close friend or family member risk from a professionalism standpoint?

Answer

It risks compromising the RD's professional objectivity due to the pre-existing personal relationship, which is a recognized conflict-of-interest and boundary concern.

#93 · Professionalism and Ethics

What is the general rule about decisions to discontinue or initiate certain medical treatments, such as parenteral nutrition, in relation to RD scope?

Answer

Such decisions typically require physician authorization even when the RD's nutrition assessment directly informs the decision, reflecting overlapping but distinct professional scopes.

#94 · Professionalism and Ethics

What should an RD do if they identify that a professional relationship with a client has become compromised?

Answer

Take corrective action, which may include setting explicit limits, consulting a supervisor or the regulatory college's practice advisory service, or appropriately terminating the relationship with continuity of care.

#95 · Professionalism and Ethics

What does the substitute decision-maker hierarchy in most provinces typically begin with?

Answer

A court-appointed guardian, followed by a named power of attorney for personal care, then family members in a specified statutory order.

#96 · Communication and Collaboration

What does the acronym OARS stand for in motivational interviewing?

Answer

Open-ended questions, Affirmations, Reflective listening, and Summaries. These are the four core communication skills used to strengthen a client's own motivation for change.

#97 · Communication and Collaboration

What is the 'righting reflex' in motivational interviewing, and why is it avoided?

Answer

The practitioner's instinct to correct or lecture a client toward a healthier choice. It is avoided because direct persuasion tends to produce sustain talk or discord (relational friction) rather than reduce the client's ambivalence about change.

#98 · Communication and Collaboration

What are the four core processes of motivational interviewing?

Answer

Engaging, focusing, evoking, and planning. These build the relationship, agree on a direction, draw out the client's own reasons for change, and develop a concrete action plan.

#99 · Communication and Collaboration

Differentiate change talk from sustain talk.

Answer

Change talk is any client statement favouring change, such as desire, ability, reason, need, or commitment. Sustain talk is any statement favouring the status quo. Skilled counselling reinforces change talk and reflects sustain talk neutrally.

#100 · Communication and Collaboration

List the five stages of the Transtheoretical (Stages of Change) model.

Answer

Precontemplation, contemplation, preparation, action, and maintenance. Counselling strategy should match the client's actual stage rather than assuming readiness for action.

#101 • Communication and Collaboration

What is health literacy?

Answer

The degree to which individuals can obtain, process, and understand basic health information needed to make appropriate health decisions. Low health literacy is linked to poorer chronic disease management independent of education level.

#102 • Communication and Collaboration

Describe the teach-back method and why it is preferred over asking 'Does that make sense?'

Answer

Teach-back asks the client to explain information back in their own words. It is preferred because it identifies real gaps in understanding, whereas 'Does that make sense?' often draws a polite but meaningless yes.

#103 • Communication and Collaboration

What reading level should most Canadian patient education materials target?

Answer

Approximately a grade 6 to 8 reading level, using plain language, short sentences, and visuals such as Canada's Food Guide plate image to support the text.

#104 • Communication and Collaboration

How does cultural safety differ from cultural sensitivity or cultural competence?

Answer

Cultural safety requires the practitioner to critically reflect on their own power, privilege, and biases, and is defined by whether the client, not the provider, experienced the care as safe and respectful of their identity and autonomy.

#105 • Communication and Collaboration

What is Indigenous food sovereignty?

Answer

The right of Indigenous communities to define and control their own food systems, including access to traditional and country foods, rather than having a mainstream Canadian dietary pattern imposed as the default standard.

#106 • Communication and Collaboration

Why should a family member generally not be used as a medical interpreter?

Answer

A family interpreter may filter, omit, or feel uncomfortable relaying sensitive health information. A professional interpreter should be used for clients with limited English or French proficiency.

#107 • Communication and Collaboration

What are the three macronutrient food group proportions shown on Canada's Food Guide plate (2019 edition)?

Answer

Roughly half vegetables and fruit, one quarter protein foods, and one quarter whole grain foods, alongside broader guidance on eating habits and water as the drink of choice.

#108 • Communication and Collaboration

What did the 2019 Canada's Food Guide revision remove compared to earlier editions?

Answer

It removed the traditional four food groups and specific serving sizes, replacing them with a plate-based visual and broader guidance on eating habits and mindful eating.

#109 • Communication and Collaboration

Define interprofessional collaboration.

Answer

Healthcare providers from different professions working together with patients, families, and communities to deliver the highest quality of care, built on role clarity, mutual respect, and reliable communication structures.

#110 • Communication and Collaboration

What determines an RD's independent scope of practice in Canada?

Answer

The provincial regulatory college where the RD is registered, such as the College of Dietitians of Ontario or an equivalent college in another province, which defines what the RD may do independently versus what requires physician involvement.

#111 • Communication and Collaboration

Which team member determines safe swallowing consistency, and how must an RD respond to that finding?

Answer

A speech-language pathologist determines texture-modified diet and thickened fluid requirements. An RD must never independently loosen a swallowing-safety restriction without a reassessment.

#112 • Communication and Collaboration

What does SBAR stand for?

Answer

Situation, Background, Assessment, Recommendation. It is a structured communication format used for concise, urgent, or handoff communication between healthcare disciplines.

#113 • Communication and Collaboration

In an SBAR communication, what does the Recommendation component specifically convey?

Answer

It states specifically what action the RD wants the other provider to take, such as starting oral nutrition supplements or ordering a repeat lab value.

#114 • Communication and Collaboration

List the four components of a SOAP note.

Answer

Subjective (client-reported information), Objective (measurable data such as weight or labs), Assessment (professional interpretation), and Plan (the specific intervention and follow-up).

#115 • Communication and Collaboration

What does ADIME stand for and what process is it built around?

Answer

Assessment, Diagnosis, Intervention, Monitoring and Evaluation. It is the documentation format built around the Nutrition Care Process used by most Canadian dietitians.

#116 • Communication and Collaboration

What is a PES statement?

Answer

The format for writing a nutrition diagnosis: Problem related to Etiology as evidenced by Signs and symptoms, for example inadequate energy intake related to nausea as evidenced by unintentional weight loss.

#117 • Communication and Collaboration

How does a nutrition diagnosis differ from a medical diagnosis?

Answer

A medical diagnosis names a disease or condition, such as colorectal cancer. A nutrition diagnosis names the specific nutrition problem the RD is intervening on, which may exist independent of the medical diagnosis.

#118 • Communication and Collaboration

What is a copy-forward error in documentation, and why is it a concern?

Answer

Copying a prior note forward without verifying it still reflects the current encounter. Regulatory colleges warn against it because it risks inaccurate, outdated information remaining in the client's active record.

#119 • Communication and Collaboration

What four qualities should all clinical documentation demonstrate regardless of the charting framework used?

Answer

Timeliness, accuracy, an objective and professional tone, and legal defensibility, since the record may be reviewed years later in a college complaint or legal proceeding.

#120 • Communication and Collaboration

How should an RD document a verbal order received from a physician about nutrition therapy?

Answer

With the date, time, and name of the ordering physician recorded clearly in the client's chart at the time the order is received.

#121 • Communication and Collaboration

When communicating a nutrition finding verbally to a physician, what should the RD lead with?

Answer

The clinical significance of the finding before the technical detail, followed by an explicit statement of the action being requested.

#122 • Communication and Collaboration

What is the recommended first step when resolving a disagreement with another team member?

Answer

Clarifying the shared goal, which in nearly every dietetic conflict is the best possible outcome for the client, to reduce defensiveness and reframe the discussion away from who is right.

#123 • Communication and Collaboration

If an RD believes a physician's diet order is unsafe, what is the RD's professional obligation?

Answer

To raise the concern directly and promptly with the ordering physician using clear clinical reasoning, rather than silently complying or silently refusing without communicating the concern.

#124 • Communication and Collaboration

What should an RD do if a safety concern about a physician's order is not resolved through direct discussion?

Answer

Document the concern and follow the facility's established escalation policy, such as involving a supervisor, medical director, or a formal team conference.

#125 • Communication and Collaboration

Why is escalating an unresolved clinical disagreement not considered a communication failure?

Answer

Because it is part of a properly functioning care system, particularly when client safety is at stake, and reflects appropriate use of established channels rather than an inability to communicate directly.

#126 • Communication and Collaboration

Why might pushing action-stage counselling strategies onto a precontemplation-stage client be counterproductive?

Answer

Because the client is not yet ready to change, and imposing specific targets or meal plans ignores their actual readiness, which can increase resistance rather than build motivation.

#127 · Communication and Collaboration

Give an example of a reflective listening statement in dietetic counselling.

Answer

Restating the meaning or feeling behind a client's statement, for example responding to 'I just don't have time to cook' with 'It sounds like finding time is the biggest barrier for you right now,' which confirms understanding and can deepen insight.

#128 · Communication and Collaboration

What is an affirmation in motivational interviewing?

Answer

A genuine acknowledgement of a client's strengths or efforts, such as recognizing consistent food log tracking, used to build confidence and rapport rather than as generic praise.

#129 • Communication and Collaboration

Why should an RD avoid assuming a single 'cultural diet' applies to every client from a broad ethnic or cultural category?

Answer

Because food practices vary widely within any broad cultural group, and assuming uniformity can lead to inaccurate assessment and care that feels disrespectful of the client's actual individual practices.

#130 • Communication and Collaboration

What health system events must culturally safe practice with Indigenous clients in Canada be understood in the context of?

Answer

The ongoing effects of colonization, residential schools, and the Sixties Scoop on food systems, trust in institutions, and health outcomes for Indigenous peoples.

#131 • Communication and Collaboration

What role do the Truth and Reconciliation Commission's Calls to Action play in dietetic education?

Answer

They explicitly name health professional education as a mechanism for addressing the ongoing harms of colonization, informing Dietitians of Canada's commitments to cultural safety and Indigenous food sovereignty.

#132 • Communication and Collaboration

What social worker collaboration areas are most relevant to dietetic practice?

Answer

Food insecurity, discharge planning to appropriate community food resources, and psychosocial barriers that make it difficult for a client to follow a nutrition plan.

#133 • Communication and Collaboration

Why is a professional, non-judgmental tone required in clinical documentation?

Answer

Because documentation must remain objective and legally defensible, and judgmental language about a client's choices can undermine professionalism and be scrutinized in a complaint or legal review.

#134 • Communication and Collaboration

Why is dietetics-specific jargon like 'PES statement' potentially problematic in verbal communication with a nurse or physician?

Answer

Because non-dietitian colleagues may not immediately recognize the term, so the RD should briefly clarify dietetics-specific language even though clinical terminology in general is often shared and understood across disciplines.

#135 · Communication and Collaboration

What is a common source of role-boundary conflict between nurses and RDs in nutrition care?

Answer

A nurse independently advising a client on carbohydrate counting without RD involvement, which may exceed the nurse's scope in that context and requires clarification of role boundaries.

#136 · Management and Leadership

What is situational leadership?

Answer

A leadership style in which the leader adapts their approach based on the competence and confidence of the team member and the demands of the task at hand. A new intern needs directive coaching, while an experienced technician benefits from a delegating style.

#137 · Management and Leadership

What is transformational leadership in a dietetics context?

Answer

A leadership approach that motivates staff through a shared vision and by modelling the values of patient-centred, evidence-based care. It is associated with improved staff engagement and retention in nutrition and food services.

#138 · Management and Leadership

What does SBAR stand for and when is it used?

Answer

Situation, Background, Assessment, Recommendation. It is a structured communication tool used to transfer critical nutrition and patient information accurately between shifts and disciplines.

#139 · Management and Leadership

Who defines what tasks an RD may delegate to a dietetic technician?

Answer

The relevant provincial regulatory college, such as the College of Dietitians of Ontario or British Columbia, defines scope of practice and what may be delegated versus what requires direct RD oversight.

#140 · Management and Leadership

Who remains accountable when an RD delegates a task to a support staff member?

Answer

The RD remains accountable for ensuring the delegate is competent to perform the delegated task, even though the task itself is carried out by the support staff member.

#141 · Management and Leadership

What are the four phases of the PDSA cycle?

Answer

Plan, Do, Study, Act. It is an iterative quality improvement framework used to test and refine small-scale practice changes before wider adoption.

#142 · Management and Leadership

What makes a quality improvement aim a SMART aim?

Answer

It is Specific, Measurable, Achievable, Relevant, and Time-bound. A SMART aim gives a quality improvement team a clear, testable target for a PDSA cycle.

#143 · Management and Leadership

What are the three categories in Donabedian's model of quality?

Answer

Structure, process, and outcome. Structure examines resources and setting, process examines whether correct steps were followed, and outcome examines the actual result for the patient or population.

#144 · Management and Leadership

Give an example of a structure measure in nutrition care quality evaluation.

Answer

Staffing ratios or the availability of therapeutic diet options are structure measures, since they describe the resources and setting available rather than the process followed or the result achieved.

#145 · Management and Leadership

Give an example of a process measure in nutrition care quality evaluation.

Answer

The percentage of high-risk patients who received a documented nutrition care plan within 24 hours of screening is a process measure, since it evaluates whether the correct steps were followed.

#146 · Management and Leadership

Give an example of an outcome measure in nutrition care quality evaluation.

Answer

Reduction in hospital length of stay or improvement in serum albumin are outcome measures, since they reflect the actual result achieved for the patient rather than the process used to get there.

#147 · Management and Leadership

What is the purpose of audit and feedback in dietetic quality improvement?

Answer

Chart audits compare documented nutrition care against an established standard, such as the Nutrition Care Process, to identify gaps. Results are fed back non-punitively, paired with education, to support improvement.

#148 · Management and Leadership

What is a key performance indicator commonly tracked in Canadian nutrition departments?

Answer

Malnutrition screening completion rate, time from referral to initial RD assessment, and patient satisfaction with meal service are all common KPIs used to monitor departmental performance over time.

#149 · Management and Leadership

What is the difference between an operating budget and a capital budget?

Answer

An operating budget covers day-to-day costs such as salaries and food, while a capital budget covers larger, longer-term purchases such as kitchen equipment, typically planned on a multi-year cycle.

#150 · Management and Leadership

How is food cost percentage calculated?

Answer

Food cost divided by food revenue, or by the total food services operating budget in a non-revenue setting, expressed as a percentage. It is a key management metric for tracking spending trends.

#151 · Management and Leadership

What is variance analysis in departmental budgeting?

Answer

Comparing actual spending against the budgeted amount for a given period to identify why a difference occurred, distinguishing a one-time event from a structural change requiring an ongoing budget adjustment.

#152 · Management and Leadership

What is a business case used for in resource allocation?

Answer

A business case is the standard tool for requesting resources from senior leadership, typically including a problem statement, options considered, projected costs and benefits, and a recommendation.

#153 · Management and Leadership

What is the difference between cost-benefit analysis and cost-effectiveness analysis?

Answer

Cost-benefit analysis expresses both costs and benefits in dollar terms, while cost-effectiveness analysis expresses benefits in clinical or functional units, such as cost per malnutrition case prevented.

#154 · Management and Leadership

Why is competency-based interviewing preferred in dietetic hiring?

Answer

Asking every candidate the same core questions tied to job-relevant competencies improves the fairness and predictive validity of hiring decisions compared to unstructured interviews.

#155 · Management and Leadership

What must be verified before finalizing an offer to a dietetic hire?

Answer

Reference checks and confirmation of current, unrestricted registration with the relevant provincial regulatory college must be completed before an offer to an RD or dietetic technician is finalized.

#156 · Management and Leadership

What does progressive discipline typically involve?

Answer

A graduated sequence: a verbal warning, a written warning, a final written warning or suspension, and, if unresolved or if misconduct is serious, termination, with documentation at every step.

#157 · Management and Leadership

What are the five rights of delegation?

Answer

The right task, right circumstance, right person, right direction and communication, and right supervision and evaluation. All five must align for a delegation decision to be appropriate.

#158 · Management and Leadership

Why should an RD not delegate an initial nutrition assessment for a complex new diagnosis to a dietetic technician?

Answer

Because that task exceeds the scope defined by most provincial colleges for a non-regulated or technician-level provider, regardless of the technician's individual experience level.

#159 · Management and Leadership

List the eight steps of Kotter's change management model.

Answer

Establish urgency, form a guiding coalition, develop a vision and strategy, communicate the vision, empower broad-based action, generate short-term wins, consolidate gains, and anchor new approaches in the culture.

#160 · Management and Leadership

What is a common cause of failed change initiatives per Kotter's model?

Answer

Skipping early steps, such as rolling out a new system without first building genuine urgency and a coalition of frontline champions, which predictably produces staff resistance.

#161 · Management and Leadership

What does the ADKAR model emphasize that Kotter's model does not focus on directly?

Answer

ADKAR (Awareness, Desire, Knowledge, Ability, Reinforcement) focuses on the individual level, reminding leaders that even a well-designed organizational change can fail if staff lack the ability to perform the new process.

#162 · Management and Leadership

List three common sources of staff resistance to change.

Answer

Fear of job loss or deskilling, lack of trust in leadership, and insufficient information about the reason for the change are all common, expected sources of resistance to organizational change.

#163 · Management and Leadership

What is a strategy to reduce staff resistance during a change initiative?

Answer

Involving affected staff early in planning, rather than only informing them after decisions are made, helps build ownership and reduces resistance compared to a top-down rollout.

#164 · Management and Leadership

What is a SWOT analysis used for in strategic planning?

Answer

A SWOT analysis examines internal strengths and weaknesses alongside external opportunities and threats, forming the environmental scan step at the start of a strategic planning process.

#165 · Management and Leadership

What is the typical time horizon for a departmental strategic plan?

Answer

A strategic plan typically covers a three-to-five-year horizon, translating a broad vision into specific goals, objectives, and action items with clear accountability and timelines.

#166 · Management and Leadership

Why should a departmental strategic plan align with organizational and regional health system priorities?

Answer

Alignment increases the likelihood that a nutrition initiative will receive sustained leadership support and funding, since it connects departmental goals to broader institutional priorities.

#167 · Management and Leadership

What is HACCP and why is it required in Canadian food service?

Answer

Hazard Analysis Critical Control Point identifies points in food production where a hazard could occur and sets critical limits, monitoring, and corrective actions. It is required under provincial food premises regulations.

#168 · Management and Leadership

What is the temperature danger zone in Canadian food safety regulation?

Answer

Potentially hazardous food must not remain between 4 and 60 degrees Celsius for longer than the time limits set by provincial food safety regulation, since bacteria multiply rapidly in this range.

#169 · Management and Leadership

What is an ENFit connector and why was it adopted?

Answer

ENFit is a feeding tube connector designed to be incompatible with intravenous line connectors, reducing the risk of enteral misconnection, a serious patient safety hazard in clinical nutrition.

#170 · Management and Leadership

What is the IDDSI framework used for?

Answer

The International Dysphagia Diet Standardisation Initiative standardizes texture-modified food and thickened liquid terminology across Canadian health care, reducing miscommunication-related adverse events.

#171 · Management and Leadership

List the hierarchy of controls from most to least effective.

Answer

Elimination, substitution, engineering controls, administrative controls, and personal protective equipment. Controls higher on the list are considered more reliable than those lower on the list.

#172 · Management and Leadership

Why is a non-slip floor surface preferred over a wet-floor warning sign?

Answer

A non-slip floor surface is an engineering control, which is more reliable long-term than a wet-floor sign, which is only an administrative control relying on staff and visitor behaviour.

#173 · Management and Leadership

What legislation governs workplace health and safety responsibilities in a Canadian food service setting?

Answer

Provincial occupational health and safety legislation sets out employer and employee responsibilities for identifying and controlling hazards in the workplace, including food service and kitchen areas.

#174 · Management and Leadership

What does the GRADE system evaluate in a clinical practice guideline?

Answer

GRADE rates the certainty of evidence supporting a recommendation as high, moderate, low, or very low, and separately rates the strength of the recommendation as strong or conditional.

#175 · Management and Leadership

What is the Knowledge-to-Action (KTA) framework?

Answer

An iterative cycle developed by Canadian researcher Ian D. Graham that describes identifying a problem, adapting knowledge to local context, assessing barriers, tailoring interventions, and evaluating and sustaining outcomes.

#176 · Management and Leadership

Why is distributing a new clinical practice guideline alone insufficient to change practice?

Answer

Translating evidence into practice requires anticipating and addressing local barriers, such as workflow or habit, and tailoring an implementation strategy rather than assuming the document alone changes behaviour.

#177 · Management and Leadership

What organizations publish Canadian clinical practice guidelines commonly used in dietetics?

Answer

Diabetes Canada and the Heart and Stroke Foundation of Canada are examples of Canadian bodies whose guidelines are prioritized over American ones in Canadian dietetic practice.

#178 · Management and Leadership

What is the purpose of closing the loop with outcome data after an evidence-based practice change?

Answer

Outcome data demonstrates impact, which sustains staff motivation for the change and provides the evidence needed to secure ongoing resources for the initiative.

#179 · Management and Leadership

What is a joint health and safety committee?

Answer

A committee required in Canadian workplaces once a minimum staff threshold is met, involving both employer and worker representatives in hazard identification, incident investigation, and safety training.

#180 · Management and Leadership

Why is punitive use of audit findings discouraged in quality improvement?

Answer

Audit findings used punitively tend to reduce staff engagement with future improvement efforts, so results are better paired with education and non-punitive process redesign.

#181 • Management and Leadership

What is the largest line item in most food service or clinical nutrition operating budgets?

Answer

Salaries and wages are typically the largest line item in a food service or clinical nutrition department's operating budget, ahead of food costs and supplies.

#182 • Management and Leadership

What does an environmental scan accomplish at the start of strategic planning?

Answer

It identifies internal strengths and weaknesses alongside external opportunities and threats, giving the planning team a realistic picture of the current situation before setting goals.

#183 · Management and Leadership

What is stakeholder engagement's role in strategic planning for a nutrition program?

Answer

Engaging patients, families, frontline staff, physicians, and community partners ensures the resulting strategic plan reflects real needs rather than assumptions made by leadership alone.

#184 · Management and Leadership

What kind of task falls outside a dietetic technician's scope of practice?

Answer

A task, such as an individualized parenteral nutrition plan for a critically ill patient, that requires direct RD assessment and sign-off rather than being delegated to a technician or support worker.

#185 · Management and Leadership

Why do daily huddles support effective team leadership in dietetics?

Answer

Daily huddles create a structured communication opportunity to share critical nutrition information and coordinate care across the team, reducing the risk of information loss between shifts.

#186 · Management and Leadership

What is the significance of psychological safety in a dietetic team?

Answer

Psychological safety means team members feel comfortable raising concerns about patient safety or workflow problems without fear of reprisal, which supports both quality improvement and staff retention.

#187 • Management and Leadership

What should happen after a small-scale test of change in a PDSA cycle shows a positive result?

Answer

In the Act phase, the team decides whether to adopt the change more broadly, adapt it further based on what was learned, or abandon it if it did not achieve the intended effect.

#188 • Management and Leadership

Why might a nutrition department fund an additional dietitian position over new equipment?

Answer

The decision should weigh evidence of impact, patient safety risk, regulatory requirements, and organizational strategic priorities, typically documented in a formal business case for leadership.

#189 · Management and Leadership

What is a risk specific to patients on modified-texture diets?

Answer

Choking or aspiration risk is a key patient safety concern for modified-texture diets, which is why standardized terminology such as IDDSI has been adopted to reduce miscommunication.

#190 · Management and Leadership

What is the role of a frontline champion in a change initiative?

Answer

A frontline champion helps build a guiding coalition and models the new behaviour for peers, increasing buy-in and reducing resistance during an organizational change rollout.

#191 · Management and Leadership

Why is documentation important throughout progressive discipline?

Answer

Documentation at every step protects the employee's right to fair process and supports the employer's ability to defend a termination decision if it is later challenged.

#192 · Management and Leadership

What does 'no surprises' mean in the context of an annual performance review?

Answer

A formal annual review should not contain surprises, because performance concerns should already have been raised informally as they arose throughout the review period.

#193 · Management and Leadership

Give an example of a common hazard requiring a hierarchy-of-controls approach in institutional food service.

Answer

Cuts from knives and slicing equipment are a common hazard; engineering controls such as blade guards are prioritized over administrative controls such as safety signage alone.

#194 · Management and Leadership

What is the relationship between a departmental KPI and a QI aim?

Answer

A KPI, such as malnutrition screening completion rate, is often the measure tracked to determine whether a QI aim, such as improving screening completion within 24 hours, has been achieved.

#195 · Management and Leadership

Why is critical appraisal of a clinical practice guideline necessary before local adoption?

Answer

Critical appraisal evaluates the quality and applicability of the guideline's evidence, using tools like GRADE, before a dietetic leader recommends it be built into local protocols.

#196 · Management and Leadership

What is a business case's role when requesting a new capital purchase?

Answer

A business case documents the problem, options considered, projected costs and benefits, and a recommendation, giving senior leadership the information needed to approve a capital budget request.

#197 · Management and Leadership

What is the risk of implementing a new documentation system without first building staff buy-in?

Answer

Skipping early change management steps such as establishing urgency and forming a coalition predictably produces passive or active staff resistance to the new system.

#198 · Management and Leadership

What does it mean for a nutrition leader to 'build evidence into existing workflows'?

Answer

Embedding a new evidence-based behaviour into existing documentation and workflow systems makes it the path of least resistance for staff, supporting sustained adoption over time.

#199 · Management and Leadership

Why do Canadian dietetic managers prioritize Canadian clinical practice guidelines over American ones?

Answer

Canadian guidelines, such as those from Diabetes Canada, reflect Canadian population health data, health system structure, and regulatory context, making them more directly applicable to Canadian practice.

#200 · Management and Leadership

What is the purpose of a chart audit against the Nutrition Care Process framework?

Answer

It identifies gaps in documented assessment, diagnosis, intervention, or monitoring and evaluation, allowing a department to target quality improvement efforts where they are most needed.

#201 • Nutrition Care

How much extra energy is recommended per day in the second trimester of pregnancy?

Answer

Approximately 340 kilocalories per day above pre-pregnancy needs. No additional energy is needed in the first trimester.

#202 • Nutrition Care

How much extra energy is recommended per day in the third trimester of pregnancy?

Answer

Approximately 450 kilocalories per day above pre-pregnancy needs, reflecting continued fetal growth and maternal tissue expansion.

#203 • Nutrition Care

What is the protein requirement during pregnancy?

Answer

1.1 grams per kilogram of body weight per day, supporting fetal tissue growth, placental development, and expanded maternal blood volume.

#204 • Nutrition Care

What daily folic acid dose does Health Canada recommend for most people planning a pregnancy?

Answer

A multivitamin containing 0.4 milligrams of folic acid daily, started before conception and continued through pregnancy to reduce neural tube defect risk.

#205 • Nutrition Care

What is the recommended dietary allowance for iron during pregnancy?

Answer

27 milligrams per day, supporting the expanded maternal red blood cell mass and fetal iron stores.

#206 • Nutrition Care

What is the recommended vitamin D intake during pregnancy?

Answer

600 international units per day, though higher supplemental amounts are often suggested given limited sunlight exposure across much of Canada.

#207 • Nutrition Care

What is the recommended weight gain range for a person with a normal pre-pregnancy BMI?

Answer

11.5 to 16 kilograms across the pregnancy, following the Institute of Medicine ranges adopted in Canadian practice.

#208 • Nutrition Care

What is the recommended weight gain range for a person with an obese pre-pregnancy BMI?

Answer

5 to 9 kilograms across the pregnancy, the smallest recommended range among the four BMI categories.

#209 • Nutrition Care

Which fish should be limited during pregnancy due to mercury advisories?

Answer

Shark, swordfish, and fresh or frozen tuna, per Health Canada mercury guidance for pregnant people.

#210 • Nutrition Care

What is the current Canadian recommendation on alcohol during pregnancy?

Answer

No amount of alcohol is recommended at any point during pregnancy, since no amount has been proven safe for fetal development.

#211 • Nutrition Care

What is the recommended caffeine limit during pregnancy?

Answer

No more than 300 milligrams per day, roughly equivalent to two 8-ounce cups of coffee.

#212 • Nutrition Care

When is gestational diabetes screening typically performed?

Answer

Between 24 and 28 weeks' gestation, using a glucose challenge or oral glucose tolerance test.

#213 • Nutrition Care

How much extra energy is recommended during the first six months of exclusive breastfeeding?

Answer

Approximately 330 kilocalories per day above pre-pregnancy needs.

#214 • Nutrition Care

What is the protein requirement during lactation?

Answer

1.3 grams per kilogram of body weight per day.

#215 • Nutrition Care

What is the joint Canadian recommendation for exclusive breastfeeding duration?

Answer

Exclusive breastfeeding for the first six months of life, with continued breastfeeding for up to two years or longer alongside complementary foods.

#216 • Nutrition Care

What vitamin D dose is recommended for breastfed infants?

Answer

400 international units per day, since breastmilk alone does not reliably meet an infant's vitamin D needs.

#217 · Nutrition Care

Why is cow's milk not recommended before 9 to 12 months of age?

Answer

It is low in iron and can contribute to gastrointestinal blood loss in young infants.

#218 · Nutrition Care

At what age is complementary feeding recommended to begin, and why?

Answer

Around six months, when developmental readiness signs appear and an infant's endogenous iron stores, built during the third trimester, begin to decline.

#219 · Nutrition Care

Name three developmental readiness signs for starting complementary foods.

Answer

Good head and neck control, the ability to sit with support, and reduced tongue-thrust reflex.

#220 · Nutrition Care

Why are iron-rich foods prioritized as first complementary foods?

Answer

Because an infant's iron stores from the third trimester of pregnancy begin declining around six months of age.

#221 • Nutrition Care

What is current Canadian guidance on introducing peanut, egg, and tree nuts to infants?

Answer

Introduce common allergenic foods in age-appropriate textures around six months rather than delaying, since early introduction reduces food allergy risk for most infants.

#222 • Nutrition Care

Why should honey be avoided before 12 months of age?

Answer

Due to the risk of infant botulism.

#223 · Nutrition Care

What type of cow's milk is recommended when transitioning a toddler at around 12 months?

Answer

Whole (3.25 percent) homogenized milk, rather than reduced-fat options, to support the higher fat needs of rapid brain development.

#224 · Nutrition Care

What does 'division of responsibility' in toddler feeding mean?

Answer

The caregiver decides what, when, and where food is offered; the child decides whether and how much to eat from what is offered.

#225 • Nutrition Care

What are the daily calcium and iron needs for toddlers aged one to three years?

Answer

Approximately 700 milligrams of calcium and 7 milligrams of iron daily.

#226 • Nutrition Care

Which growth chart standard does Canada use for children up to age 19?

Answer

The World Health Organization growth charts, adopted by the Canadian Paediatric Society.

#227 · Nutrition Care

What growth chart pattern should prompt further nutrition assessment in a child?

Answer

Crossing of major percentile lines on weight-for-age, height-for-age, or BMI-for-age curves over time.

#228 · Nutrition Care

What is the peak daily calcium requirement during adolescence, and why?

Answer

1,300 milligrams per day, to support accrual of peak bone mass, since much of adult bone mineral density is established during the teenage years.

#229 · Nutrition Care

Why do menstruating adolescents have increased iron needs?

Answer

To replace iron lost through menstrual bleeding, on top of the increased needs of the pubertal growth spurt.

#230 · Nutrition Care

Name four common nutrition concerns to screen for in adolescents.

Answer

Disordered eating, iron deficiency, inadequate calcium and vitamin D intake, and the nutritional demands of competitive sport participation.

#231 • Nutrition Care

What does the ABCD nutrition assessment framework stand for?

Answer

Anthropometric, Biochemical, Clinical, and Dietary assessment, the four integrated categories of a structured nutrition assessment.

#232 • Nutrition Care

Give three examples of anthropometric measurements used in nutrition assessment.

Answer

Height or length, weight, and mid-upper arm circumference (body mass index and waist circumference are also examples).

#233 • Nutrition Care

What percentage of unintentional weight loss over one month is considered clinically significant?

Answer

A loss of more than 5 percent of usual body weight in one month.

#234 • Nutrition Care

What percentage of unintentional weight loss over six months is considered clinically significant?

Answer

A loss of more than 10 percent of usual body weight in six months.

#235 · Nutrition Care

Why should serum albumin and prealbumin be interpreted cautiously as nutrition markers?

Answer

Both are influenced heavily by inflammation and hydration status, not purely by nutritional status.

#236 · Nutrition Care

What is a nutrition-focused physical examination used for?

Answer

To systematically evaluate physical signs consistent with specific macronutrient or micronutrient deficits, complementing biochemical data.

#237 · Nutrition Care

Name three methods used in dietary assessment.

Answer

A 24-hour recall, a food frequency questionnaire, and a multi-day food record.

#238 · Nutrition Care

Besides intake amount, what other factors does dietary assessment consider?

Answer

Food security status, cultural and religious food practices, socioeconomic constraints, food preparation resources, and personal food preferences.

#239 · Nutrition Care

What is the purpose of distinguishing an intake problem from a clinical problem in a nutrition diagnosis?

Answer

It directs an individualized nutrition care plan by identifying whether the root cause is inadequate intake, a disease process, or a behavioural-environmental factor.

#240 · Nutrition Care

What protein intake range is often recommended for healthy older adults?

Answer

1.0 to 1.2 grams per kilogram of body weight per day, higher than the general adult recommendation, to help preserve muscle mass.

#241 · Nutrition Care

Define sarcopenia.

Answer

The progressive loss of skeletal muscle mass and strength associated with aging, contributing to frailty, falls, and reduced functional independence.

#242 · Nutrition Care

What two components are needed together to address sarcopenia effectively?

Answer

Adequate protein intake distributed across meals, particularly leucine-rich sources, combined with resistance exercise.

#243 • Nutrition Care

Name a validated malnutrition screening tool developed specifically for Canadian hospitals.

Answer

The Canadian Nutrition Screening Tool.

#244 • Nutrition Care

Name a validated nutrition screening tool widely used in community and long-term care settings for older adults.

Answer

The Mini Nutritional Assessment.

#245 • Nutrition Care

Name four factors that increase malnutrition risk in older adults.

Answer

Reduced appetite, changes in taste and smell, dentition problems, and social isolation (polypharmacy and chronic disease also contribute).

#246 • Nutrition Care

What framework does Canadian practice use to standardize texture-modified diets and thickened liquids?

Answer

The International Dysphagia Diet Standardisation Initiative (IDDSI) framework, using a common numbered and coloured scale.

#247 · Nutrition Care

Why must dietitians balance texture modification with nutritional adequacy in dysphagia management?

Answer

Overly restrictive texture modification can reduce food intake and worsen malnutrition, so texture-modified diets must remain as nutritious and palatable as possible.

#248 · Nutrition Care

What professional typically works closely with dietitians on dysphagia management?

Answer

A speech-language pathologist, who performs swallowing assessments and guides texture recommendations alongside the dietitian.

#249 • Nutrition Care

Why is vitamin B12 deficiency common in older adults?

Answer

Age-related decline in intrinsic factor and stomach acid production reduces absorption of vitamin B12 from food.

#250 • Nutrition Care

Why are older adults at increased risk of dehydration?

Answer

A blunted thirst response and reduced total body water increase dehydration risk with advancing age.

#251 • Nutrition Care

Which growth reference does the Public Health Agency of Canada endorse for children birth to 19 years?

Answer

The World Health Organization growth charts, reflecting growth patterns of healthy children raised in optimal conditions.

#252 • Nutrition Care

Who jointly develops the Dietary Reference Intakes used in Canadian dietetic practice?

Answer

Health Canada and the United States National Academies.

#253 · Nutrition Care

What does the Recommended Dietary Allowance (RDA) represent?

Answer

A Dietary Reference Intake value set to meet the nutrient needs of nearly all healthy individuals in a specific life stage and sex group.

#254 · Nutrition Care

What does the Tolerable Upper Intake Level (UL) represent?

Answer

The highest average daily nutrient intake level unlikely to pose risk of adverse health effects for almost all individuals.

#255 · Nutrition Care

What model does Canada's Food Guide use, and what did it replace?

Answer

A plate-based model emphasizing proportions of vegetables and fruits, protein foods, and whole grain foods, replacing the older food groups and servings model.

#256 · Nutrition Care

What is the recommended beverage of choice in Canada's Food Guide?

Answer

Water.

#257 · Nutrition Care

How does iodine need change during lactation?

Answer

It remains elevated to support thyroid hormone transfer into breastmilk.

#258 · Nutrition Care

What is a key strategy for managing nausea and vomiting of pregnancy?

Answer

Small frequent meals, ginger, and avoidance of individual trigger foods.

#259 · Nutrition Care

Why is iron-fortified infant formula recommended over standard cow's milk in the first year?

Answer

Cow's milk lacks adequate iron and can irritate the infant gut, contributing to blood loss, unlike iron-fortified formula designed for infant needs.

#260 · Nutrition Care

What common food safety practice applies to deli meats and pates during pregnancy?

Answer

They should be heated to steaming before eating, or avoided, due to listeria risk.

#261 • Nutrition Care

What is the calcium requirement for most non-pregnant, non-lactating adults, for comparison to pregnancy needs?

Answer

1,000 milligrams per day, the same as during pregnancy, since fetal skeletal mineralization is supported partly by maternal bone turnover rather than solely by higher intake.

#262 • Nutrition Care

What clinical sign might suggest a micronutrient deficiency on nutrition-focused physical examination?

Answer

Glossitis or angular stomatitis, which can reflect deficiencies such as B vitamins or iron.

#263 • Nutrition Care

Why is mid-upper arm circumference a useful anthropometric measure in some populations?

Answer

It provides a nutrition status estimate that is less affected by fluid shifts than body weight and can be measured when height or weight is difficult to obtain.

#264 • Nutrition Care

What overall principle guides interpretation of biochemical data in nutrition assessment?

Answer

Biochemical values must be interpreted in the context of the whole clinical picture, never in isolation.

#265 • Nutrition Care

Why is dysphagia screening usually the first step before a full swallowing assessment?

Answer

It quickly identifies individuals at risk who need a more detailed swallowing assessment, often performed by nursing or speech-language pathology staff.

#266 • Nutrition Care

What percentage reduction in A1C is typically achieved through medical nutrition therapy delivered by a registered dietitian in diabetes?

Answer

Roughly 1.0 to 2.0 percent, depending on diabetes type and duration, per Diabetes Canada's 2023 Clinical Practice Guidelines.

#267 · Nutrition Care

What is the primary macronutrient determinant of postprandial glucose response in diabetes management?

Answer

Total carbohydrate quantity, which is why carbohydrate counting is a core MNT teaching tool, especially for intensive insulin therapy.

#268 · Nutrition Care

What daily fibre intake does Diabetes Canada recommend, and which fibre type is emphasized?

Answer

15 grams per 1000 kilocalories, emphasizing viscous soluble fibre from oats, psyllium, barley, and legumes for glycemic and lipid benefit.

#269 · Nutrition Care

Describe the 15-15 rule for treating hypoglycemia.

Answer

Give 15 grams of fast-acting carbohydrate, wait 15 minutes, then recheck blood glucose, repeating if still under 4.0 mmol/L.

#270 · Nutrition Care

What carbohydrate distribution pattern is recommended for gestational diabetes, and why is breakfast unique?

Answer

Consistent carbohydrate across three meals and two to three snacks; breakfast carbohydrate tolerance is often lower due to elevated cortisol.

#271 • Nutrition Care

What are the four components of the Portfolio Diet?

Answer

Viscous soluble fibre, plant sterols (2 g/day), soy protein replacing animal protein, and nuts, particularly almonds.

#272 • Nutrition Care

How much LDL cholesterol reduction can the Portfolio Diet achieve, and what is it comparable to?

Answer

A 20 to 30 percent reduction, comparable to a starting-dose statin.

#273 · Nutrition Care

What sodium restriction does the DASH pattern recommend for hypertension management?

Answer

Less than 2000 mg of sodium per day.

#274 · Nutrition Care

What fluid and sodium restrictions are typical for heart failure management?

Answer

Fluid restriction of roughly 1.5 to 2 litres per day and sodium restriction to less than 2000 mg per day, tighter in advanced disease.

#275 • Nutrition Care

What nutrition approach is used for cardiac cachexia in advanced heart failure?

Answer

A shift toward energy- and protein-dense, smaller, more frequent meals rather than further dietary restriction.

#276 • Nutrition Care

Which framework is used to stage chronic kidney disease for nutrition therapy in Canada?

Answer

The KDIGO staging framework, based on estimated glomerular filtration rate (eGFR).

#277 · Nutrition Care

What protein intake is recommended for hemodialysis patients, and why is it elevated?

Answer

1.0 to 1.2 g/kg/day or higher, elevated to offset amino acid losses that occur during dialysis.

#278 · Nutrition Care

Why do peritoneal dialysis patients typically need even higher protein intake than hemodialysis patients?

Answer

Protein is continuously lost into the dialysate fluid, requiring 1.2 to 1.3 g/kg/day, while fluid restriction is generally more liberal.

#279 · Nutrition Care

Why is phosphorus binder use typically required with meals for hemodialysis patients?

Answer

Dialysis removes phosphorus far less efficiently than functioning kidneys do, so serum phosphorus rises without binders.

#280 · Nutrition Care

How should potassium restriction be individualized in CKD?

Answer

Based on trends in serum potassium rather than a fixed universal restriction, since unnecessary restriction reduces diet quality without benefit.

#281 • Nutrition Care

What nutrition focus shifts occur after a successful renal transplant?

Answer

Higher protein and energy needs support wound healing immediately post-transplant, while long-term care shifts to cardiovascular risk reduction and monitoring for post-transplant diabetes, with prior restrictions usually liberalized.

#282 • Nutrition Care

What diet approach is typically used during an acute flare of inflammatory bowel disease?

Answer

A low-residue or low-fibre diet to reduce mechanical irritation and symptom burden, liberalized during remission.

#283 • Nutrition Care

Why is vitamin B12 deficiency especially common in Crohn's disease affecting the terminal ileum?

Answer

Vitamin B12 is absorbed in the terminal ileum, so disease or resection there impairs absorption.

#284 • Nutrition Care

What dietary restriction is required for celiac disease, and why is cross-contamination a concern?

Answer

Strict, lifelong avoidance of gluten from wheat, barley, and rye; even small cross-contamination amounts trigger autoimmune villous damage.

#285 • Nutrition Care

Why might a newly diagnosed celiac disease patient need temporary lactose restriction?

Answer

Villous damage reduces lactase-producing enterocytes, so lactose intolerance is common until the intestinal lining recovers.

#286 • Nutrition Care

What does the acronym FODMAP stand for in the low FODMAP diet for irritable bowel syndrome?

Answer

Fermentable oligosaccharides, disaccharides, monosaccharides, and polyols.

#287 · Nutrition Care

What are the three phases of the low FODMAP diet?

Answer

Strict elimination for two to six weeks, systematic reintroduction of FODMAP subgroups, and personalization to a sustainable long-term diet.

#288 · Nutrition Care

What guiding principle determines the choice between enteral and parenteral nutrition?

Answer

"If the gut works, use it": enteral nutrition is preferred whenever the gastrointestinal tract is functional.

#289 · Nutrition Care

Why is enteral nutrition generally preferred over parenteral nutrition when the gut is functional?

Answer

It maintains gut mucosal integrity and gut-associated immune function, and carries lower infectious complication risk and cost than parenteral nutrition.

#290 · Nutrition Care

When is post-pyloric or jejunal feeding preferred over gastric feeding?

Answer

When gastric feeding carries a high aspiration risk.

#291 • Nutrition Care

When are semi-elemental or elemental enteral formulas indicated?

Answer

For patients with malabsorption or severe pancreatic insufficiency, since these formulas require less digestive capacity.

#292 • Nutrition Care

What is refeeding syndrome, and what electrolyte shifts characterize it?

Answer

A dangerous metabolic response to reintroducing carbohydrate in a severely malnourished patient, causing intracellular shifts of phosphate, potassium, and magnesium with resulting hypophosphatemia, hypokalemia, and hypomagnesemia.

#293 · Nutrition Care

How is refeeding syndrome prevented when starting nutrition support in a high-risk patient?

Answer

Starting at a conservative caloric level (often 10 to 20 kcal/kg/day), correcting electrolytes before and during refeeding, and supplementing thiamine before carbohydrate delivery.

#294 · Nutrition Care

Why is thiamine given before carbohydrate delivery in refeeding syndrome prevention?

Answer

To prevent Wernicke's encephalopathy, since carbohydrate metabolism depletes thiamine reserves in a depleted patient.

#295 • Nutrition Care

When is parenteral nutrition indicated instead of enteral nutrition?

Answer

When the gastrointestinal tract is non-functional, inaccessible, or perforated, or needs cannot be met enterally.

#296 • Nutrition Care

What access is required for long-term or hypertonic parenteral nutrition formulations?

Answer

Central venous access, due to the higher osmolarity of long-term parenteral formulations.

#297 · Nutrition Care

How does cancer cachexia differ from simple starvation?

Answer

It is a multifactorial wasting syndrome involving systemic inflammation and altered metabolism with progressive muscle loss that nutrition support alone cannot fully reverse.

#298 · Nutrition Care

What is a common consequence of head and neck radiation therapy affecting nutrition intake?

Answer

Severe mucositis, xerostomia (dry mouth), and dysphagia, which may persist long after treatment ends.

#299 · Nutrition Care

What screening tool was developed specifically for malnutrition risk in oncology patients?

Answer

The Patient-Generated Subjective Global Assessment (PG-SGA).

#300 · Nutrition Care

When should enteral nutrition be initiated in a hemodynamically stable, critically ill ICU patient?

Answer

Within 24 to 48 hours of ICU admission, per Canadian Clinical Practice Guidelines for critical care nutrition.

#301 • Nutrition Care

Why has routine gastric residual volume monitoring been de-emphasized in current ICU practice?

Answer

Routine checks do not reliably predict aspiration risk and may cause unnecessary feeding interruptions; monitoring clinical signs of intolerance is preferred.

#302 • Nutrition Care

What is permissive underfeeding in critical care nutrition, and why might it be used early in illness?

Answer

Deliberately providing less than full estimated energy needs in the acute early phase, since evidence suggests this may reduce complications compared with early full feeding.

#303 • Nutrition Care

What protein range is often targeted in critically ill patients, and why is it elevated?

Answer

1.2 to 2.0 g/kg/day or higher, to offset the profound catabolism seen in conditions like major trauma, burns, and sepsis.

#304 • Nutrition Care

How does a true food allergy differ mechanistically from a food intolerance?

Answer

A food allergy is immune-mediated, often IgE-mediated, and can cause anaphylaxis; a food intolerance does not involve the immune system and is typically dose-dependent.

#305 • Nutrition Care

What are Health Canada's priority food allergens that must be declared on packaged food labels?

Answer

Peanuts, tree nuts, milk, eggs, fish, shellfish, wheat, soy, sesame, and mustard, along with sulphites at declared thresholds.

#306 • Nutrition Care

What causes lactose intolerance, and how does its management typically differ from a food allergy's management?

Answer

Insufficient lactase enzyme activity; management usually involves limiting rather than fully eliminating dairy, since small amounts and hard cheese or yogurt are often tolerated.

#307 · Nutrition Care

How does non-celiac gluten sensitivity differ from celiac disease?

Answer

It produces symptoms with gluten exposure without the autoimmune villous damage of celiac disease, and celiac disease must be ruled out first.

#308 · Nutrition Care

Why does warfarin therapy require consistent, rather than low, vitamin K intake?

Answer

Fluctuating vitamin K intake destabilizes the INR used to monitor anticoagulation; the goal is dietary consistency, not vitamin K avoidance.

#309 • Nutrition Care

What food restriction is required with monoamine oxidase inhibitors (MAOIs), and why?

Answer

Avoidance of tyramine-rich aged, fermented, and cured foods, because accumulated tyramine can precipitate a hypertensive crisis when monoamine oxidase is inhibited.

#310 • Nutrition Care

How does grapefruit juice interact with certain medications?

Answer

It inhibits intestinal cytochrome P450 3A4 enzyme activity, increasing blood levels of medications metabolized through this pathway, such as certain statins and calcium channel blockers.

#311 • Nutrition Care

Why should dairy products be separated in timing from tetracycline or fluoroquinolone antibiotics?

Answer

These antibiotics chelate with calcium and other divalent cations, reducing antibiotic absorption when taken together.

#312 • Nutrition Care

What nutrition monitoring concerns arise with long-term corticosteroid use?

Answer

Sodium retention, hyperglycemia, increased protein catabolism, and increased calcium and vitamin D needs to offset steroid-induced bone loss.

#313 · Nutrition Care

What is the risk of combining potassium-sparing diuretics with potassium supplements or potassium-containing salt substitutes?

Answer

An increased risk of hyperkalemia.

#314 · Nutrition Care

What electrolyte losses are associated with loop and thiazide diuretics?

Answer

Potassium and magnesium loss, which may require monitoring and supplementation.

#315 • Nutrition Care

What eating patterns does Diabetes Canada endorse rather than a single prescriptive diet?

Answer

Mediterranean-style, DASH-style, vegetarian or vegan, and lower-glycemic-index patterns, provided each meets individual preferences and glycemic targets.

#316 • Nutrition Care

How much additional A1C improvement can choosing lower-glycemic-index carbohydrates provide?

Answer

Approximately 0.2 to 0.5 percent, beyond carbohydrate quantity management alone.

#317 • Nutrition Care

How often should fatty fish be consumed for cardiovascular risk reduction, per Heart and Stroke Foundation guidance?

Answer

Twice weekly, as a source of omega-3 fatty acids.

#318 • Nutrition Care

What protein target is appropriate for earlier-stage CKD (stages 1 to 3a, eGFR above 45)?

Answer

Moderate protein intake around 0.8 g/kg/day, avoiding excessive restriction that could compromise nutritional status.

#319 • Nutrition Care

How is fluid restriction typically set for a hemodialysis patient?

Answer

Based on interdialytic weight gain targets, often limited to roughly 1000 mL plus urine output.

#320 • Nutrition Care

What deficiencies should be screened for in inflammatory bowel disease due to malabsorption and increased losses?

Answer

Iron, vitamin B12, vitamin D, and zinc.

#321 • Nutrition Care

What deficiencies are common at celiac disease diagnosis due to years of malabsorption?

Answer

Iron, folate, and vitamin D deficiencies.

#322 • Nutrition Care

What nutrition strategy may help manage short bowel syndrome and severe malabsorption?

Answer

A combination of oral rehydration solutions, soluble fibre to slow transit, and in severe cases, parenteral nutrition support.

#323 · Nutrition Care

Why does a standard nasogastric tube generally suit only short-term enteral feeding?

Answer

It is typically appropriate for feeding durations under four to six weeks, with longer-term needs served by a PEG tube.

#324 · Nutrition Care

What complication risks does long-term parenteral nutrition carry beyond central line infection?

Answer

PN-associated liver disease and metabolic complications requiring monitoring of blood glucose, triglycerides, and liver function.

#325 • Nutrition Care

What practical nutrition strategies help manage cancer treatment side effects like nausea and mucositis?

Answer

Small, frequent, energy- and protein-dense meals, texture modification, antiemetic strategies for nausea, and oral nutrition supplements when intake alone is insufficient.

#326 • Nutrition Care

What does radiation enteritis from abdominal or pelvic radiation typically cause?

Answer

Diarrhea and malabsorption.

#327 • Nutrition Care

Why is indirect calorimetry preferred over predictive equations for estimating energy needs in critical illness when available?

Answer

It provides a more individualized measurement of actual energy expenditure than population-based predictive equations.

#328 • Nutrition Care

What documentation and scope-of-practice standard do provincial regulatory colleges require of Canadian dietitians?

Answer

Nutrition care plans must be documented, evidence-informed, and within the dietitian's individual scope of competence, escalating or consulting when a case exceeds that scope.

#329 · Nutrition Care

What is the underlying nutrition care philosophy that connects assessment, guideline knowledge, and monitoring across clinical conditions?

Answer

Individualized application of the nutrition care process, translating evidence-based guidelines into a person's real dietary pattern while monitoring and adjusting as clinical status changes.

#330 · Nutrition Care

Why is dietary consistency, rather than low intake, the correct approach to vitamin K for a patient on warfarin?

Answer

Warfarin works by antagonizing vitamin K; stable day-to-day intake keeps the INR predictable, while fluctuating intake causes unpredictable swings in anticoagulation.

#331 • Population Health Promotion

What year was Canada's current Food Guide released, and what model replaced food groups and servings?

Answer

2019. The plate-based model replaced the old four food groups and specific serving counts.

#332 • Population Health Promotion

In the 2019 Canada's Food Guide plate model, what proportion of the plate is vegetables and fruits?

Answer

Half the plate, with protein foods and whole grain foods each filling one quarter.

#333 · Population Health Promotion

What beverage does the 2019 Canada's Food Guide promote as the beverage of choice?

Answer

Water.

#334 · Population Health Promotion

Beyond the plate, what three behavioural themes does the 2019 Food Guide emphasize?

Answer

Being mindful of eating habits, cooking more often, and eating meals with others, alongside using food labels.

#335 · Population Health Promotion

Why did Health Canada develop the 2019 Food Guide without direct food industry consultation on core recommendations?

Answer

To reduce conflict of interest, following criticism that earlier editions were influenced by industry input.

#336 · Population Health Promotion

What is household food security defined as?

Answer

Physical and economic access at all times to sufficient, safe, and nutritious food that meets dietary needs for an active, healthy life.

#337 · Population Health Promotion

What tool does Statistics Canada use to measure household food insecurity, and how many items does it contain?

Answer

The Household Food Security Survey Module, an 18-item validated tool.

#338 · Population Health Promotion

What survey embeds the Household Food Security Survey Module for national data collection?

Answer

The Canadian Community Health Survey, and since 2019 also the Canadian Income Survey.

#339 · Population Health Promotion

What are the four classification levels of the Household Food Security Survey Module?

Answer

Food secure, marginally food insecure, moderately food insecure, and severely food insecure.

#340 · Population Health Promotion

What distinguishes moderately food insecure from severely food insecure households?

Answer

Moderate involves compromised quality or quantity of food; severe involves reduced food intake and disrupted eating patterns, including hunger.

#341 · Population Health Promotion

What research program based at the University of Toronto produces national food insecurity monitoring data?

Answer

PROOF.

#342 · Population Health Promotion

Approximately what proportion of Canadian households experiences some level of food insecurity, per national monitoring?

Answer

Roughly one in four households, per the most recent PROOF/Canadian Income Survey data, a rate that has risen for several consecutive years.

#343 · Population Health Promotion

Which household types have substantially higher food insecurity rates in Canada?

Answer

Lone-parent female-led households, households relying on social assistance, Black and Indigenous households, and households in the territories.

#344 · Population Health Promotion

Is food insecurity in Canada fundamentally a food supply problem or an income problem?

Answer

An income and financial resource problem, not primarily a food supply or food literacy problem.

#345 · Population Health Promotion

Why do food banks not resolve food insecurity at a population level?

Answer

They do not address the underlying lack of income, and food insecurity persists even where food banks are well used.

#346 · Population Health Promotion

Name two income-based policy levers linked to reduced food insecurity in Canadian research.

Answer

The Canada Child Benefit and adequate income assistance or minimum wage rates.

#347 · Population Health Promotion

What is a basic income guarantee, in the context of food insecurity policy debate?

Answer

A proposed income policy providing a minimum guaranteed income, associated in research with reductions in food insecurity where implemented.

#348 · Population Health Promotion

What error does a dietitian make by treating food insecurity as a food literacy problem alone?

Answer

It misattributes an income-driven problem to lack of knowledge, and food education alone does not resolve inadequate income.

#349 · Population Health Promotion

List at least five social determinants of health relevant to nutrition status in Canada.

Answer

Income and income distribution, education, employment and working conditions, food insecurity, housing, early childhood development, Indigenous status, and social exclusion.

#350 · Population Health Promotion

What explains more variation in population health outcomes: health behaviours or the social determinants of health?

Answer

The social determinants of health explain far more variation in health outcomes than health behaviours or health care access alone.

#351 · Population Health Promotion

Define nutrition equity.

Answer

The principle that dietetic practice and policy should work to close health gaps arising from unjust and avoidable social conditions, rather than treating all populations identically.

#352 · Population Health Promotion

What is intervention-generated inequity?

Answer

When a universal intervention delivered without attention to differential access benefits those already advantaged more than those with greater need, widening disparities.

#353 · Population Health Promotion

Name three strategies a dietitian can use to design programs with an equity lens.

Answer

Removing cost barriers, culturally tailored messaging, and community co-design rather than one-size-fits-all programming.

#354 · Population Health Promotion

What are the two data sources typically combined in a community nutrition needs assessment?

Answer

Quantitative data such as survey and program utilization data, and qualitative data such as community consultation and key informant interviews.

#355 · Population Health Promotion

What does the PRECEDE-PROCEED model do?

Answer

It works backward from desired health and quality-of-life outcomes through behavioural, environmental, and educational diagnoses to identify appropriate interventions.

#356 · Population Health Promotion

What four elements does a logic model map in program planning?

Answer

Inputs, activities, outputs, and short, medium, and long-term outcomes.

#357 · Population Health Promotion

Distinguish process evaluation from outcome evaluation.

Answer

Process evaluation measures whether a program was implemented as intended, including reach and fidelity; outcome evaluation measures whether it achieved its intended effects.

#358 · Population Health Promotion

Distinguish formative evaluation from summative evaluation.

Answer

Formative evaluation occurs during program development to refine design; summative evaluation occurs after the program runs to judge overall effectiveness.

#359 · Population Health Promotion

Why should evaluation indicators be specified at the planning stage, before a program launches?

Answer

So baseline data exists for comparison, allowing a meaningful before-and-after or comparative evaluation.

#360 · Population Health Promotion

What is the dominant health promotion framework taught in Canadian public health nutrition?

Answer

The socio-ecological model.

#361 · Population Health Promotion

List the five nested levels of the socio-ecological model.

Answer

Individual, interpersonal, organizational or institutional, community, and public policy.

#362 · Population Health Promotion

Why does individual-level nutrition education alone rarely produce lasting population-level change?

Answer

Because behaviour is shaped by environments and policies that individuals cannot control through knowledge or willpower alone.

#363 · Population Health Promotion

Give an example of a community-level intervention under the socio-ecological model addressing childhood obesity.

Answer

Improving affordable produce access in the neighbourhood food environment or municipal zoning limiting fast food outlet density.

#364 · Population Health Promotion

Where and when was the Ottawa Charter for Health Promotion developed?

Answer

In Canada in 1986, at the first International Conference on Health Promotion.

#365 · Population Health Promotion

List the five action areas of the Ottawa Charter for Health Promotion.

Answer

Building healthy public policy, creating supportive environments, strengthening community action, developing personal skills, and reorienting health services.

#366 · Population Health Promotion

Define Indigenous food sovereignty.

Answer

The right of Indigenous peoples to define their own food and agriculture systems and determine their own food and land use policies, centring self-determination and cultural connection.

#367 · Population Health Promotion

How does food sovereignty differ from food security?

Answer

Food security focuses on access to sufficient food, while food sovereignty centres self-determination and control over the food system itself, not only nutrient adequacy.

#368 · Population Health Promotion

What role do traditional and country foods play for many First Nations, Inuit, and Métis communities?

Answer

They are nutritionally dense and tied to cultural identity, land-based knowledge transmission, spiritual practice, and community relationships.

#369 · Population Health Promotion

Name three barriers to traditional food access in Indigenous communities.

Answer

Climate change altering migration and harvest patterns, environmental contaminants such as mercury, and the high cost of hunting and fishing equipment.

#370 · Population Health Promotion

What is Nutrition North Canada?

Answer

The federal subsidy program intended to improve access to perishable healthy food and support traditional harvesting in isolated northern communities without year-round road, rail, or marine access.

#371 · Population Health Promotion

What criticisms have been raised about Nutrition North Canada?

Answer

Inadequate transparency in how subsidies reach consumers, insufficient support for traditional harvesting, and persistently high food costs despite the subsidy.

#372 · Population Health Promotion

What professional stance should a dietitian take when working with Indigenous communities on food sovereignty?

Answer

Practise cultural humility, support Indigenous-led initiatives, and recognize food sovereignty as a distinct determinant of health from food security.

#373 · Population Health Promotion

Under which dietetic practice competency roles do policy and advocacy work fall in Canada?

Answer

Policy developer and advocate.

#374 · Population Health Promotion

Give an example of a municipal-level nutrition policy lever.

Answer

Zoning that limits fast food outlet density near schools, or a municipal food charter or food policy council.

#375 · Population Health Promotion

What is the National School Food Program, announced in the 2024 federal budget?

Answer

Canada's first national school food policy framework, intended to be implemented in partnership with provinces, territories, and Indigenous partners.

#376 · Population Health Promotion

What federal regulation on food packaging took effect for Canadian manufacturers beginning in 2026?

Answer

Front-of-package labelling for foods high in sodium, sugars, and saturated fat.

#377 · Population Health Promotion

What is the core role of a provincial regulatory college for dietitians, such as the College of Dietitians of Ontario?

Answer

Public protection through registration, standards of practice, and discipline.

#378 · Population Health Promotion

What is the core role of Dietitians of Canada, distinct from a provincial regulatory college?

Answer

Advocating for the profession and for nutrition policy; membership is voluntary and distinct from provincial regulatory obligations.

#379 · Population Health Promotion

Does a Registered Dietitian's legal scope of practice flow from Dietitians of Canada membership or from the provincial college?

Answer

From the provincial regulatory college, not from Dietitians of Canada membership.

#380 · Population Health Promotion

What is the prevention paradox in population-level chronic disease prevention?

Answer

A small risk reduction applied across a whole population often prevents more disease burden than an intensive intervention applied only to a small high-risk group.

#381 • Population Health Promotion

Where does most dietary sodium in the Canadian diet come from?

Answer

Processed and prepared foods, not the salt shaker.

#382 • Population Health Promotion

Why does population-level sodium reduction target the food supply rather than individual counselling alone?

Answer

Because most sodium intake comes from processed and prepared foods, so reformulating the food supply reaches more people than counselling individuals.

#383 · Population Health Promotion

Which two Canadian organizations provide the reference clinical practice guidelines for diabetes and cardiovascular disease prevention?

Answer

Diabetes Canada and the Heart and Stroke Foundation of Canada.

#384 · Population Health Promotion

Why should a Canadian dietitian apply Diabetes Canada guidelines over American Diabetes Association guidelines?

Answer

Because screening thresholds, target values, and terminology sometimes differ between Canadian and American guidance.

#385 · Population Health Promotion

Give an example of a built environment intervention for chronic disease prevention.

Answer

Improving access to physical activity infrastructure and healthy food retail in a neighbourhood.

#386 · Population Health Promotion

Define nutrition surveillance.

Answer

The ongoing, systematic collection, analysis, and interpretation of population-level nutrition and health data used to monitor trends and guide policy.

#387 · Population Health Promotion

What is the primary national data source for population health and nutrition surveillance in Canada?

Answer

The Canadian Community Health Survey, conducted by Statistics Canada.

#388 · Population Health Promotion

What dietary intake data does the Canadian Community Health Survey's nutrition component collect?

Answer

24-hour dietary recall data, used to estimate population dietary intake against reference standards such as the Dietary Reference Intakes.

#389 · Population Health Promotion

What does the Canadian Health Measures Survey add beyond self-reported questionnaire data?

Answer

Direct physical measurements such as height, weight, and blood biomarkers, overcoming limitations of self-reported data alone.

#390 · Population Health Promotion

What surveillance source tracks food insecurity at the population level in Canada?

Answer

The Household Food Security Survey Module, embedded in the Canadian Community Health Survey and Canadian Income Survey.

#391 · Population Health Promotion

How does surveillance differ from a one-time needs assessment or research study?

Answer

Surveillance is ongoing and population-focused, while a needs assessment or research study is time-limited and often more localized.

#392 · Population Health Promotion

How does surveillance data typically prompt a new public health nutrition program?

Answer

Surveillance documenting a worsening trend, such as rising food insecurity in a subgroup, is often the starting point that prompts a new program or policy response.

#393 · Population Health Promotion

What is a key criticism of relying on charitable food banks as the main response to food insecurity?

Answer

Food banks do not address the underlying income shortfall and capture only a fraction of food-insecure households.

#394 · Population Health Promotion

What does the 2019 Food Guide recommend regarding protein food sources?

Answer

Choosing plant-based protein sources such as legumes, tofu, and nuts more often, alongside fish, lower-fat dairy, and lean meats.

#395 · Population Health Promotion

How does the 2019 Food Guide support institutional and policy use beyond individual guidance?

Answer

It underpins institutional menu standards in schools, correctional facilities, and long-term care, and informs food subsidy and procurement policy.

#396 · Population Health Promotion

Why is the 2019 Food Guide's plate model intentionally generic rather than food-specific?

Answer

So it can be adapted to diverse food cultures while keeping the underlying proportions of vegetables and fruit, protein, and whole grains consistent.

#397 · Population Health Promotion

What territory has recorded food insecurity rates exceeding half of all households in some survey years?

Answer

Nunavut.

#398 · Population Health Promotion

What organization publishes the Auditor General findings criticizing Nutrition North Canada's transparency?

Answer

The Auditor General of Canada.

#399 · Population Health Promotion

What is a healthy public policy, per the Ottawa Charter action area?

Answer

Policy at any level of government, such as municipal, provincial, or federal, that supports population health, such as zoning or food labelling law.

#400 · Population Health Promotion

What is meant by community action in the Ottawa Charter framework?

Answer

Strengthening community action, meaning empowering communities to set priorities and take action on their own health, rather than only receiving top-down programming.

#401 · Population Health Promotion

What does reorienting health services mean under the Ottawa Charter?

Answer

Shifting health services toward prevention and health promotion rather than treatment alone.

#402 · Population Health Promotion

Why is coalition work with organizations like Dietitians of Canada important for effective advocacy?

Answer

It combines professional evidence and credibility with broader civil society reach, and directs advocacy to the relevant level of government.

#403 · Population Health Promotion

What is a food policy council?

Answer

A municipal-level body that brings together stakeholders to develop coordinated local food policy, such as a food charter.

#404 · Population Health Promotion

How does marketing restriction legislation aimed at children function as a chronic disease prevention tool?

Answer

It limits children's exposure to marketing of foods and beverages high in sodium, sugars, or saturated fat, reducing a driver of poor dietary patterns at a population level.

#405 · Population Health Promotion

What is the purpose of front-of-package labelling for high sodium, sugars, or saturated fat?

Answer

To make products high in these nutrients immediately identifiable to consumers at the point of purchase.

#406 · Population Health Promotion

Why might a sugar-sweetened beverage tax be proposed as a population health measure?

Answer

As a policy lever to reduce consumption of sugar-sweetened beverages across a population, complementing labelling and marketing restrictions.

#407 · Population Health Promotion

What is meant by dose and fidelity in process evaluation?

Answer

Dose refers to how much of the program participants actually received; fidelity refers to whether the program was delivered as originally designed.

#408 · Population Health Promotion

Why is a needs assessment that combines both quantitative and qualitative data considered stronger than either alone?

Answer

Administrative and survey data show patterns but may miss context, while community consultation can surface lived-experience priorities that data alone cannot capture.

#409 · Population Health Promotion

What does it mean for a determinant of health to be described as avoidable and unjust in the context of nutrition equity?

Answer

It means the resulting health disparity arises from unfair social conditions that could be changed through policy, not from unavoidable biological or personal factors.

#410 · Population Health Promotion

Give an example of institutional-level influence on eating behaviour under the socio-ecological model.

Answer

A school's food policy or a workplace cafeteria's menu offerings.

#411 · Population Health Promotion

What organizational bias might arise if a dietitian relies only on food bank usage statistics to measure community food insecurity?

Answer

Food bank usage captures only a fraction of food-insecure households, so it underestimates true population food insecurity.

#412 · Population Health Promotion

What is the relationship between income policy and nutrition policy in a population health context?

Answer

Income policy, such as minimum wage or income assistance rates, functions as a nutrition policy lever because food insecurity is primarily an income problem.

#413 · Population Health Promotion

Why is multi-level intervention, addressing several socio-ecological levels at once, generally more effective than single-level intervention?

Answer

Multi-level interventions address both individual behaviour and the environments and policies shaping that behaviour, producing larger and more sustained effects.

#414 · Population Health Promotion

What is an example of a provincial-level nutrition policy lever?

Answer

A provincial school food program or a provincial poverty reduction strategy.

#415 · Population Health Promotion

How did residential schools and forced relocation affect Indigenous food systems?

Answer

They disrupted traditional food systems, with measurable negative effects on nutrition status and overall wellbeing that continue today.

#416 · Population Health Promotion

What environmental contaminant is a known barrier to traditional food harvesting in some northern regions?

Answer

Mercury contamination in some fish and wildlife.

#417 · Population Health Promotion

Why is it important for a dietitian to specify evaluation indicators before a community program launches, rather than after?

Answer

Baseline data collected before launch is required to make a meaningful comparison of outcomes after the program runs.

#418 · Population Health Promotion

What is a logic model used for beyond initial program planning?

Answer

It is also used for evaluation, since it maps the expected chain from activities to outputs to short, medium, and long-term outcomes that evaluation can then measure.

#419 · Population Health Promotion

Why does the CDRE expect candidates to distinguish nutrition surveillance from a single research study?

Answer

Because choosing the right data source for a population health scenario depends on knowing whether ongoing population monitoring or a time-limited localized study is appropriate.

#420 · Population Health Promotion

What common thread links Canada's Food Guide, front-of-package labelling, and school food programs as population health tools?

Answer

All three are population-level interventions that shape the food environment and policy context rather than relying solely on individual education.

#421 · Food Provision

What is a master cycle menu?

Answer

A repeating sequence of menus, commonly 3 to 6 weeks long, designed to provide variety while allowing predictable procurement, staffing, and production planning.

#422 · Food Provision

What edition of Canada's Food Guide is current for institutional menu planning, and what is its key structural feature?

Answer

The 2019 edition, which uses a plate-based visual model instead of the older food groups and servings system.

#423 · Food Provision

What is a non-selective menu?

Answer

A menu format where every resident on a given diet order receives the same meal, with no choice of entree, which lowers cost and production complexity but may reduce satisfaction.

#424 · Food Provision

What is a selective menu?

Answer

A menu format offering a choice of two or more entrees at each meal, which tends to improve satisfaction and intake but increases production complexity and food cost.

#425 · Food Provision

What is an 'always available' or 'alternative' list in long-term care menu planning?

Answer

A short standing set of items, such as a sandwich, soup, or omelette, that residents can request instead of the scheduled menu, supporting choice without full a la carte complexity.

#426 · Food Provision

Why should menu evaluation include plate waste studies, not just nutrient analysis?

Answer

A menu can meet every nutrient target on paper yet be poorly accepted by residents, leading to low actual intake; plate waste and resident feedback capture real-world consumption.

#427 · Food Provision

What role does a registered dietitian typically play in long-term care menu approval?

Answer

The RD reviews and signs off on the cycle menu at set intervals, often annually or with any substantive change, to confirm it meets nutrition standards and regulatory requirements.

#428 · Food Provision

What is a therapeutic diet manual?

Answer

A reference document, approved by the facility's medical advisory committee and reviewed by an RD, defining every diet order that can be prescribed, its nutrient parameters, and permitted or restricted foods.

#429 · Food Provision

Why must a diet manual specify an exact sodium target rather than a vague term like 'low salt'?

Answer

Vague descriptors create ambiguity in production and monitoring; an exact milligram target gives kitchen staff and clinicians an unambiguous, measurable standard to follow.

#430 · Food Provision

What are common therapeutic diets used in Canadian institutional practice?

Answer

Regular, diabetic or carbohydrate-consistent, renal, cardiac or sodium-restricted, low-fibre, high-protein/high-calorie, and fluid-restricted diets, among others.

#431 • Food Provision

What should a registered dietitian do when a diet order is clinically inappropriate for a resident's overall status?

Answer

The RD should communicate with the prescriber to clarify or recommend liberalization, such as when a strict renal diet is ordered for a resident who is underweight and at risk of malnutrition.

#432 • Food Provision

What is the function of a computerized dietary software system in diet order management?

Answer

It links the diet order, allergies, texture and fluid level, and preferences into one profile and generates a tray ticket or pick list for kitchen production.

#433 · Food Provision

What federal legislation sets requirements for federally regulated food businesses in Canada?

Answer

The Safe Food for Canadians Act and its regulations, supported by guidance from the Canadian Food Inspection Agency and Health Canada.

#434 · Food Provision

At what level of government are institutional kitchen inspections typically enforced?

Answer

At the provincial and municipal level, through public health units enforcing food premises regulations.

#435 · Food Provision

What does HACCP stand for?

Answer

Hazard Analysis and Critical Control Point, a systematic approach to identifying and controlling food safety hazards.

#436 · Food Provision

List the seven core principles of HACCP.

Answer

Conduct a hazard analysis, identify critical control points, establish critical limits, establish monitoring procedures, establish corrective actions, establish verification procedures, and establish record-keeping and documentation.

#437 · Food Provision

What is a critical control point in HACCP?

Answer

A specific step in the food handling process where a hazard can be prevented, eliminated, or reduced to an acceptable level, such as cooking to a validated temperature.

#438 · Food Provision

What is the generally recognized temperature danger zone in Canadian food safety guidance?

Answer

4 to 60 degrees Celsius, the range in which bacteria multiply most rapidly in potentially hazardous foods.

#439 · Food Provision

What is the maximum combined time a time-temperature control for safety food should remain in the danger zone?

Answer

A combined total of 4 hours, or a shorter facility-specific limit, from production through service.

#440 · Food Provision

What is the required cooling window for hot cooked food?

Answer

From 60 to 20 degrees Celsius within 2 hours, and from 20 to 4 degrees Celsius within a further 4 hours, for a maximum total of 6 hours.

#441 · Food Provision

What internal temperature must reheated food reach for hot holding, and within what time frame?

Answer

At least 74 degrees Celsius within 2 hours of the start of reheating.

#442 · Food Provision

What are the minimum standards for hot holding and cold holding of food?

Answer

Hot holding must maintain food at or above 60 degrees Celsius; cold holding must maintain food at or below 4 degrees Celsius.

#443 · Food Provision

Why does placing a large batch of hot food into a large, deep container before refrigeration create a food safety risk?

Answer

A large deep container slows heat transfer, so the centre of the food stays in the temperature danger zone longer than the allowed cooling window, allowing bacterial growth.

#444 · Food Provision

What is the correct storage order for raw proteins in a walk-in cooler?

Answer

Ready-to-eat foods are stored above raw proteins, and raw proteins are stored from lowest required cooking temperature on top to highest (poultry and stuffed items) on the bottom.

#445 · Food Provision

Name Health Canada's recognized priority food allergens.

Answer

Peanuts, tree nuts, milk, egg, soy, wheat and other gluten-containing cereals, sesame, fish, crustaceans and molluscs, mustard, and sulphites.

#446 · Food Provision

What is the purpose of colour-coded cutting boards in institutional food service?

Answer

To physically separate raw and ready-to-eat foods and prevent cross-contamination during preparation.

#447 · Food Provision

What is a standardized recipe and why is it essential?

Answer

A recipe specifying exact ingredient quantities, method, portion size, and yield; it links menu planning, procurement, nutrient analysis, and cost control together consistently.

#448 · Food Provision

What does the FIFO principle mean in inventory management?

Answer

First-in, first-out: older stock must be used before newer stock of the same item to minimize spoilage and waste.

#449 · Food Provision

What is a par level in institutional food service inventory?

Answer

The minimum quantity of an item that should be on hand before reordering, set based on delivery lead time, cycle demand, and a safety margin.

#450 · Food Provision

What should staff check when receiving a refrigerated or frozen food delivery?

Answer

Quantity, quality, and temperature against the purchase order and specification, using a calibrated thermometer, and reject deliveries showing temperature abuse or damage.

#451 · Food Provision

What is a group purchasing organization in Canadian institutional food service?

Answer

An arrangement where multiple health regions or facilities combine purchasing volume to negotiate lower unit costs from vendors.

#452 · Food Provision

Why should a large-batch recipe be tested at actual production volume rather than mathematically scaled up from a small recipe?

Answer

Scaling changes surface-area-to-volume ratio, cooking time, seasoning perception, and equipment behaviour, so mathematically extrapolated recipes often perform differently at true volume.

#453 · Food Provision

What quality control methods are used at multiple points in institutional food production?

Answer

Sensory evaluation of appearance, aroma, texture, and taste; portion control using standardized tools; and plate presentation checks.

#454 · Food Provision

What is a cook-chill production system?

Answer

A system that prepares food in bulk, rapidly chills it under controlled time-temperature parameters, and transports it to satellite sites for reheating and service.

#455 · Food Provision

Why does cook-chill/cook-freeze production require especially rigorous HACCP adherence?

Answer

The extended time between production and service, and multiple transfer points, create more opportunities for temperature abuse if not carefully controlled.

#456 · Food Provision

What is IDDSI?

Answer

The International Dysphagia Diet Standardisation Initiative, the national Canadian framework for describing texture-modified foods and thickened liquids on one common scale.

#457 · Food Provision

How many levels does the IDDSI continuum have, and what range do they span?

Answer

Eight levels, numbered 0 through 7, spanning both drinks and foods on a single scale from thin liquids to regular unmodified food.

#458 · Food Provision

What does IDDSI level 0 represent?

Answer

Thin liquids, the least modified fluid consistency on the IDDSI continuum.

#459 · Food Provision

What does IDDSI level 4 represent?

Answer

Extremely thick liquids or pureed foods, a consistency that cannot be drunk from a cup or eaten with a fork.

#460 · Food Provision

What does IDDSI level 6 represent?

Answer

Soft and bite-sized food, requiring some chewing ability but no cutting with a knife.

#461 · Food Provision

What does IDDSI level 7 represent?

Answer

Regular, easy to chew, or regular unmodified food, the least modified food texture on the continuum.

#462 · Food Provision

What is the IDDSI flow test used for?

Answer

A bedside test using a 10-millilitre syringe to measure how much liquid remains after a fixed number of seconds, used to classify fluid thickness levels objectively.

#463 · Food Provision

What are the fork drip test and fork pressure test used for?

Answer

Bedside tests used to classify texture-modified foods consistently by observing how food behaves under gentle pressure or dripping from a fork.

#464 · Food Provision

Why did Canada adopt IDDSI instead of facility-specific terms like 'minced' or 'mechanical soft'?

Answer

Facility-specific terms varied inconsistently between institutions, creating serious risk when a resident transferred between facilities; IDDSI provides one standardized national scale.

#465 · Food Provision

What nutritional risk is strongly associated with pureed and thickened-fluid diets?

Answer

Reduced palatability and acceptance, leading to higher risk of malnutrition and dehydration if not managed with nutrient-dense preparations and monitoring.

#466 · Food Provision

Who typically determines the appropriate IDDSI level for a resident, and what is the RD's role?

Answer

A speech-language pathologist assesses swallowing and recommends the level; the RD translates this into food and fluid production and monitors nutrient and fluid adequacy.

#467 · Food Provision

What sustainability practice increases plant-based protein options in menu planning?

Answer

Deliberately incorporating more legumes, tofu, and other plant proteins into cycle menus, aligned with Canada's Food Guide's emphasis on plant-based choices.

#468 · Food Provision

Name three points in food service operations where food waste commonly occurs.

Answer

Over-production, plate waste from residents or patients, and spoilage from poor inventory rotation.

#469 · Food Provision

How is over-production food waste best addressed?

Answer

Through better forecasting and more accurate portion planning based on historical demand and census.

#470 · Food Provision

How is plate waste best addressed in institutional food service?

Answer

By improving menu acceptability and ensuring portions are appropriately sized for the population served.

#471 · Food Provision

What is a food waste diversion program?

Answer

A program that directs unavoidable food waste to composting rather than landfill disposal.

#472 · Food Provision

What are the core requirements of halal food preparation?

Answer

Meat from animals slaughtered according to Islamic law, and avoidance of pork and alcohol in preparation and ingredients.

#473 · Food Provision

What is required for kosher food preparation?

Answer

Separation of meat and dairy, avoidance of certain species and preparation methods, often using pre-packaged products certified by a recognized kosher authority.

#474 · Food Provision

Why can a simple vegetarian or vegan diet plan be nutritionally inadequate if not carefully designed?

Answer

Removing animal products without planning can leave gaps in protein, iron, vitamin B12, and omega-3 fatty acid intake; the diet must be planned for full nutrient adequacy.

#475 · Food Provision

What is the recommended approach for identifying cultural and religious dietary needs at intake?

Answer

Asking residents or patients respectfully and specifically about their preferences and requirements, rather than assuming based on apparent background.

#476 · Food Provision

Why is cross-contact prevention important for halal and kosher accommodation?

Answer

Shared equipment that has contacted non-permitted foods can compromise the religious status of an accommodated meal even if the ingredients themselves are correct.

#477 · Food Provision

What is a diet manual's role as a communication bridge?

Answer

It translates a prescribing clinician's diet order into unambiguous nutrient parameters and food restrictions that food service staff can consistently produce.

#478 · Food Provision

What does 'time-temperature control for safety food' mean?

Answer

A category of potentially hazardous foods, such as cooked meats, dairy, cooked rice, and cut melon, that requires strict time and temperature control to prevent pathogen growth.

#479 · Food Provision

What certified training is commonly required for food handlers in Canadian institutional kitchens?

Answer

A recognized food handler certification course, such as FOODSAFE or an equivalent provincial program.

#480 · Food Provision

What is the difference between a perpetual and a periodic inventory system?

Answer

Perpetual inventory tracks stock continuously as items are received and issued, while periodic inventory relies on physical counts taken at set intervals.

#481 · Food Provision

Why is portion control important beyond cost management?

Answer

It ensures nutritional consistency, so that every resident on a given order receives the same intended nutrient content, not just the same cost per serving.

#482 · Food Provision

What is the significance of resident councils in long-term care menu planning?

Answer

They give residents a formal channel to provide feedback on menus and food service, supporting person-centred care and improving acceptance and intake.

#483 · Food Provision

What minimum internal cooking temperature is commonly cited for poultry in Canadian food safety guidance?

Answer

74 degrees Celsius, a widely used critical limit to eliminate pathogens such as Salmonella.

#484 · Food Provision

What is the purpose of verification procedures within a HACCP plan?

Answer

To confirm the overall system is functioning as intended, such as through periodic calibration of thermometers or independent review of monitoring records.

#485 · Food Provision

Why must record-keeping be part of a HACCP system?

Answer

It creates an auditable trail demonstrating that critical limits were monitored and corrective actions were taken when needed.

#486 · Food Provision

What is the recommended action if a critical limit is not met during monitoring?

Answer

A predefined corrective action must be taken, such as discarding the food or reprocessing it, following the facility's HACCP plan.

#487 · Food Provision

How does cultural food accommodation relate to quality of life for long-term care residents?

Answer

Food is closely tied to identity and comfort, and may be one of the few areas where residents can exercise meaningful autonomy and preference in their daily routine.

#488 · Food Provision

What is a product specification in procurement?

Answer

A written document defining the exact quality, size, and packaging requirements a vendor must meet for a given food item.

#489 · Food Provision

What safety concern arises specifically with texture-modified thickened fluids if under-thickened?

Answer

Fluids that are thinner than prescribed increase the risk of aspiration in residents with dysphagia.

#490 · Food Provision

Why should institutional menus plan for variety of colour, texture, and temperature within each meal?

Answer

Sensory variety improves resident satisfaction and acceptance, which supports better actual nutrient intake.

#491 · Food Provision

What is the role of Diabetes Canada guidelines in institutional diet manuals?

Answer

They provide the Canadian evidence-based reference for carbohydrate-consistent and diabetic diet parameters used in therapeutic diet manuals.

#492 · Food Provision

What is the role of Heart and Stroke Foundation guidance in cardiac diet planning?

Answer

It provides the Canadian evidence-based reference for sodium restriction and cardiovascular-protective dietary parameters used in institutional cardiac diets.

#493 · Food Provision

Why should a renal diet order be reassessed if a resident develops unintended weight loss?

Answer

Strict protein or nutrient restriction can worsen malnutrition risk, so the diet may need liberalization to prioritize adequate intake over strict biochemical control.

#494 · Food Provision

What is meant by 'plate waste study' as a menu evaluation tool?

Answer

A structured observation of how much food residents or patients actually leave uneaten, used to identify menu items with poor acceptance.

#495 • Food Provision

Why should raw meat never be stored above ready-to-eat food in a cooler?

Answer

Drip or leakage from raw meat could contaminate ready-to-eat food stored below it, so ready-to-eat items must be stored on higher shelves.

#496 • Food Provision

What is the main advantage of local and seasonal food sourcing in institutional food service?

Answer

It reduces transportation-related environmental impact and supports the local economy, while often improving freshness.

#497 · Food Provision

What is the purpose of a tray ticket or electronic pick list in food service?

Answer

It communicates the exact diet order, texture level, allergies, and preferences for an individual resident or patient to kitchen and tray-line staff at the point of assembly.

#498 · Food Provision

Why is seasoning intensity a concern when scaling a recipe to large batch volume?

Answer

Some seasonings intensify while others become diluted relative to perception as volume increases, so seasoning must be adjusted and tested at true production scale.

#499 · Food Provision

What should a registered dietitian confirm before approving an accommodated cultural or religious menu?

Answer

That the accommodated menu remains nutritionally adequate and that the accommodation is genuinely implemented in day-to-day production, not just documented on paper.

#500 · Food Provision

What does a facility's therapeutic diet manual need to define for every diet order?

Answer

Its nutrient parameters, permitted and restricted foods, and any required texture or fluid modifications, in clear and measurable terms.